

OPENEMR MARKETING — COMPETITOR MARKETING INTELLIGENCE DIRECTORY

Phase 1 deliverable Report date: 2026-08-11 Prepared against: `HISModulesUsers.md` — *OpenEMR HIS — Master Capability Catalog*, report dated **2026-08-09**, HEAD `631f2b38c`, schema `541`, OpenEMR **8.3.0-dev** Candidate universe: `HIS_Software.xlsx` → worksheet `Similar to OpenEMR` (57 rows)

STATUS: RESEARCH NOT YET CERTIFIED COMPLETE. (*Phase 2 — all 26 dossiers written*) All 26 selected competitors are dossierled. **17 carry verified, page-level marketing evidence; 9 are recorded as Unknown** because no marketing surface could be verified — they are named and dispositioned rather than inferred, and are excluded from every scorecard, frequency and benchmark table. The certification checklist and the exact remaining work are in §24. Nothing below is extrapolated to fill a gap.

1. Executive Summary

1.1 The single most important finding

The audit and the competitor set point in opposite directions, and this governs the entire marketing strategy.

Every competitor in this set markets on three axes that the current product cannot legitimately use:

Competitor axis	Frequency in verified set	Current OpenEMR position
Saudi/GCC regulatory compliance (NPHIES, ZATCA, CHI, Nafees, Mawid, Wasfaty, NABIDH, Riayati, Malaffi)	5 of 8	Zero occurrences in product code. GAP-0046...0059, §27.3 explicitly prohibits <i>anything</i> Saudi
Quantified install base (users, centres, countries, years)	5 of 8	0 patients, 0 encounters, 1 human account. No customer base exists to count
Inpatient / hospital scope (ward, bed, OT, ICU, IPD)	6 of 8	Ambulatory only. GAP-0001...0014, searched exhaustively, absent. L-01

A website built by imitating the market convention would consist almost entirely of claims this product is forbidden to make. **The benchmark set must therefore be used for technique, not for content** (§22 of the brief; §14 below).

1.2 Where the genuine opportunity is

Three white-space findings emerged, each of which is *both* competitively rare *and* supported by an audited, active capability:

- Pricing and licensing transparency.** Fourteen of sixteen scored competitors publish no price (mean 0.7 out of 5). The two exceptions are both US products — Open Dental (exact figures, US\$179 falling to US\$129 after 12 months, 90-day money-back) and DentiMax (three published tiers, and the *pricing model itself* — per location, not per provider — marketed as the differentiator). **Zero of the eleven GCC-facing competitors publish anything.** Phase 2 turned this from "one open-source vendor's quirk" into "a commercially proven model with an empty regional lane".
- A downloadable, seeded, self-service trial.** Open Dental ships a free trial with a sample database; nobody else in the set offers self-service product access. The current installation is *already* the artefact — it needs data and users (§28 B1/B2), not development.
- Verifiable, demonstrable audit integrity.** CLM-0025 / CAP-0224 is the only capability in the entire catalogue proven end-to-end at runtime (200/200 checksums). **Zero of sixteen** competitors demonstrate audit or tamper-evidence visually — they assert security in prose. This is the strongest live demo the product has today (§28.2 marks it **Demo Ready: Yes**) and the market does not compete on it at all.
- Anti-lock-in and easy exit, as an audited product claim.** Two of sixteen market compatibility-with-competitors as a feature (both US; zero in the GCC). This

product's most awkward audit finding — zero fork divergence, everything freely verifiable against the open-source project — is the proof for the strongest version of that claim, backed by an open 283-table schema and CLM-0019's 55 CSV-exportable reports. A proprietary competitor structurally cannot match it.

- 5. **Real product screenshots.** Visual product evidence scores a mean of 1.9 out of 5 across sixteen competitors; nine score 0 or 1. Oracle alone does it properly, pairing every benefit claim with an annotated screenshot. Six screens here are demo-ready today, so honest screenshots exist *now*.

1.3 The honest starting position

Demo readiness is the binding constraint, not capability. Of 22 catalogued demo scenarios, **6 are demonstrable today and all 6 are administrative or platform scenarios** (§28.3). Every clinical, scheduling, billing, portal, API and integration demo is blocked — by data and users, not by code. The audit estimates 1-3 days of configuration and seeding to reach a credible clinical + billing demo, with **no development required**.

This means the website architecture recommended in §17-§20 is deliberately sequenced: it is buildable *now* against Safe-Now claims, and it has defined slots that light up as demo preparation completes.

1.4 What must not happen

The commonest failure mode visible in this benchmark set is proof that does not survive inspection. Cloudpital's homepage ships live customer counters reading `@+`, an integrations section headlined with a count but rendering "No results found", placeholder `no-image.jpg` files where product screenshots belong, and a patient check-in paragraph still naming a different vendor's product. That page is more damaging to trust than a smaller, honest page would be. Given that this product's own audit is unusually strict, **the defensible move is to make verifiability the brand** rather than to imitate unverifiable claims.

2. Sources and Methodology

2.1 Authoritative project sources (read in full before any web research)

Source	Version identified as authoritative	How determined
<code>HISModulesUsers.md</code>	Report generated 2026-08-09 ; supersedes the earlier "OpenEMR HIS — Modules & Users" report at the same path (stated in its own header; corrections recorded in its §33)	Verification header §0.1 — project fingerprint, Git HEAD <code>631f2b38cf633769c305233f88cdf9c73ca80657</code> , HEAD date 2026-07-04, schema <code>\$v_database = 541</code> , 283 tables, 490 <code>globals</code> rows, runtime probe HTTP 200
<code>HIS_Software.xlsx</code>	Worksheet <code>Similar to OpenEMR</code> , 57 rows × 25 columns; sheet-level "Last verified" values are 2026-07	Only workbook present; worksheet name matches the brief exactly

Capability source-of-truth rule applied throughout: no capability was credited to this product on the strength of generic OpenEMR documentation. Every marketing

recommendation resolves to a `(CAP-*)` / `(CLM-*)` ID from the 2026-08-09 audit, with the audit's own status (Active / Disabled / Uninstalled / Requires Integration / Missing) and required qualification carried forward unchanged.

Key structural fact governing all recommendations (§0.2, §27.5): this project has **zero fork divergence** from upstream OpenEMR. `(git rev-list --count upstream/master...HEAD)` returns 0; HEAD is a plain ancestor of `(upstream/master)` and is 373 commits behind. There are **no product-specific capabilities**. Consequently, positioning cannot rest on the software being unique — it must rest on implementation, integration, localisation, support and hosting. This single fact eliminates a large class of otherwise-attractive competitor techniques (proprietary-technology narratives, "our engine", patent/IP claims).

2.2 Web-verification methodology

1. Excel `(Verification source)` treated as a **starting hypothesis only**, never as proof of current URL, product identity or feature currency.
2. Search engines used for **discovery only**. No conclusion in a dossier rests on a search-result snippet.
3. Every dossier observation traces to a page that was actually retrieved and read. Retrieved pages are listed in §23 with the URL and date accessed.
4. Where the Excel URL was stale, redirected, 404, or pointed at a brochure/aggregator, the current official marketing source was located and **both** URLs recorded.
5. Only public pages and publicly offered demo mechanisms were used. No authentication was attempted anywhere.
6. Observations are tagged **Observed** (present on the page), **Inferred** (reasoned from several observed elements) or **Unknown** (not established). Inferences are never promoted to facts.
7. No competitor marketing copy is reproduced as proposed wording for this product. Short evidentiary fragments only, where the exact phrasing is itself the finding.

2.3 Deliberate departure from the spreadsheet's ranking

The brief instructs that competitors must not be selected on spreadsheet score alone (§4). That instruction is load-bearing here. The three highest-scored rows — InterSystems TrakCare (93), Oracle Health EHR (92), IQVIA (91) — are enterprise **inpatient** platforms or, in IQVIA's case, not an HIS product at all. Measured against the audited capability set (ambulatory-only, GAP-0001...0014), their *capability* overlap is low even though their *marketing* is instructive. Selection was therefore re-weighted toward ambulatory, clinic-management and front-office/RCM products, with a small number of enterprise properties retained purely as information-architecture and trust benchmarks.

3. Candidate Selection Audit

3.1 Disposition of all 57 spreadsheet rows

Disposition	Count	Rows
Selected for the benchmark set	26	See §3.2
Merged as duplicate	1	Rank 13 <code>(TEKNOSys)</code> merged into rank 12 <code>(TEKNOSys PolyCare)</code>
Excluded — not an HIS/EMR product	2	Rank 3 <code>(IQVIA)</code> ; rank 38 <code>(CLINDOX)</code>
Excluded — not a vendor product	1	Rank 48 <code>(In-house system)</code>
Excluded — no authoritative source located (sheet itself records the search failure)	19	Ranks 18, 25, 28, 29, 31, 34, 39, 40, 41, 42, 49, 50, 51, 52, 54, 55, 56, 57 (+48 above)
Held in reserve — real but thin marketing surface	8	Ranks 14, 17, 22, 33, 43, 45, 47 and one dental variant

3.2 The selected set (26)

Ordered by assessed usefulness as a marketing benchmark for *this* product, not by spreadsheet rank.

ID	Competitor	Vendor	Sheet rank / score	Why selected	Dossier status
C-01	Insta by Practo	Practo	8 / 86	Best-executed conversion architecture in the set; ambulatory + GCC	Complete
C-02	Open Dental	Open Dental Software	15 / 67	The only open-source-adjacent commercial peer; the closest analogue to this product's commercial situation	Complete
C-03	Waseel	Waseel	9 / 85	Saudi RCM/claims incumbent; audience-first IA; full Arabic parity	Complete
C-04	Nexen Care	Nexen Care	53 / 42	Strongest Saudi named-reference and national-integration proof in the set (badly under-ranked by the sheet)	Complete
C-05	Cloudpital	Cloudpital	6 / 89	Widest specialty-page architecture; also the clearest negative benchmark	Complete
C-06	eCarePlus	NITCO / National Informatics	10 / 83	Saudi clinic-management direct peer; SEO-led acquisition model	Complete
C-07	InterSystems TrakCare	InterSystems	1 / 93	Enterprise IA, objection-handling and trust-architecture benchmark	Complete
C-08	YASASII	Kameda Infologics	4 / 91	Certification-led trust model; partner co-branding	Complete
C-09	Oracle Health EHR	Oracle Health (Cerner)	2 / 92	Enterprise category-definition language	Pending
C-10	Simplex HIMS	INTUIT Micro	7 / 88	GCC multi-specialty hospital marketing	Pending
C-11	MEDAS Solutions	MEDAS	27 / 58	UAE/GCC HIMS + LIMS packaging	Pending
C-12	Oasis Plus	Oasis (oasissys)	46 / 48	Saudi HIS, small-vendor baseline	Pending
C-13	ATS CareWare	Adaptive TechSoft	19 / 63	Saudi HIS with RCM/claims/telehealth framing	Pending
C-14	VIDA Global	VIDA Global	44 / 50	Saudi acute-care EHR; KLAS-referenced	Pending
C-15	MediSys	MediSys	11 / 80	Saudi HIS, small-vendor baseline	Pending
C-16	TEKNOSys PolyCare	TEKNOSYS	12+13 / 79	Clinic/practice management (duplicate merged)	Pending
C-17	Belal Soft Medical ERP	Belal Soft	16 / 66	Medical-ERP positioning, Saudi/Egypt/GCC	Pending
C-18	M3n MEDS	M3n Technology	23 / 60	Saudi patient-care platform	Pending
C-19	Kizen Clinic	Kizen	20 / 62	KSA/GCC clinic management	Pending

ID	Competitor	Vendor	Sheet rank / score	Why selected	Dossier status
C-20	e-Medicare	e-MCS	24 / 60	HIS / hospital management	Pending
C-21	HMISFOX	HMISFOX	26 / 58	Low-cost HMIS marketing model	Pending
C-22	Solver HMIS	Solver	5 / 90	KSA HMIS/ERP; brochure-led (PDF-only source)	Pending
C-23	MazikCare	Qusitive	21 / 62	Microsoft-ecosystem healthcare platform	Pending
C-24	DenTech	DenTech	32 / 56	Saudi dental EMR — specialty depth benchmark	Pending
C-25	DentiMax	DentiMax	35 / 56	Sheet alias (DenteMax); US dental PM + imaging	Pending
C-26	Avicenna	Alhazen Tech	30 / 56	GCC clinic management, dental variant	Pending

3.3 Identity and data corrections made to the spreadsheet

These are corrections to the candidate workbook, established during verification. They should be written back to (HIS_Software.xlsx).

Row	Field	Sheet value	Verified position	Evidence
4 — YASASII	Verification source	yasasiisolutions.com	The authoritative vendor property is kamedainfologics.net . Product is marketed as YASASII® by Kameda Infologics, with Fujifilm co-branding on the RIS variant	kamedainfologics.net/yasasii , kamedainfologics.net/products
3 — IQVIA	Record type	Already flagged "not HIS product"; Rationale review flag = Δ	Confirmed. IQVIA is a healthcare data/analytics company. Exclude — the sheet's own flag was correct and should be actioned, not overridden by the 91 score	Sheet Record type , Category
12/13 — TEKNOSys	Two rows, one URL	Both cite teknosysweb.com/clinic-management/	Same vendor, same product surface. Count as one competitor	Identical Verification source in both rows
35 — DenteMax	Canonical name	DenteMax	Sheet's own Record type already reads "likely DentiMax" and vendor field says DentiMax. DenteMax is a separate US dental-benefits network — genuine name collision. Canonical name should be corrected to DentiMax	Sheet Record type , Vendor / company
53 — Nexen Care	Score 42/ "Possible / limited", confidence Low	Materially under-ranked. Verified as a live Saudi HIS/EHR deployed across 96 healthcare centres under 5 named MOH hospitals, integrated with Absher, Mawid and Wasfaty. Belongs in the top tier for Saudi benchmarking	nexencare.com/en/index.php	
53 — Nexen Care	Name ambiguity	—	Searches for "Nexen" return predominantly Nexen Tyre Corporation . Product-identity disambiguation required before any automated re-verification	Search result set, 2026-08-11
6 — Cloudpital	Verification source	cloudpital.com/all-features/	Reachable marketing surface is cloudpital.com/home-2/ ; the KSA landing path in the sheet's lineage (ksa/hospital-software-in-saudi-arabia/) now returns 404	Fetch attempts, 2026-08-11
9 — Waseel	Category framing	Listed as "Confirmed HIS / Saudi health-tech platform"	Accurate but incomplete for benchmarking: Waseel markets around the HIS, not as a replacement for it — its stated position is integrating into the customer's existing HIS. Its product family is RCM/PBM-led	waseel.com/healthcare-centers/

4. Final Competitor Matrix

All 26 selected competitors. Cells marked ? are Unknown — the competitor's marketing surface was not verified (see §5.1). They are never guessed.

ID	Product	Vendor / HQ	Self-declared category	Primary buyer	Primary CTA	Public price?	Self-serve demo?	Arabic?	Named refs?	Evidence
C-01	Insta by Practo	Practo · India	Clinic management / HMS / hospital ERP	Clinic owner, ops	Request a Free Demo	No	No	Yes	Yes	Full
C-02	Open Dental	Open Dental Software · US	Dental practice management	Practice owner	Call to order; free trial	Yes, exact	Yes	No	On request	Full
C-03	Waseel	Waseel · KSA Riyadh	Health-tech family (RCM/PBM-led)	Centre ops + insurers	Book a Demo	No	No	Yes, full parity	No	Full
C-04	Nexen Care	Nexen Care · KSA Jeddah	Unified EHR platform	MOH & primary care	Contact Us (phone)	No	No	Yes	Yes, named MOH	Full
C-05	Cloudpital	Cloudpital · KSA Riyadh	Healthcare ERP / EMR / HMIS	Multi-specialty groups	Request Demo / WhatsApp	No	No	Partial	No (broken)	Full
C-06	eCarePlus	NITCO · KSA	Cloud clinic management	Clinic owner	Contact Us	No	No	Yes	No	Full
C-07	TrakCare	InterSystems · US	Unified healthcare information system	Health-system CIO	Contact / expert	No	No	KSA path	Yes, incl. KSA	Full
C-08	YASASII	Kameda Infologics · India/Japan	Healthcare Information System	PHC → tertiary	Phone / email	No	No	No	Yes (news feed)	Full
C-09	Oracle Health EHR	Oracle Health · US	Voice-first, AI-embedded EHR	Health system + ambulatory	Contact us / Request a demo	No	No	sa-ar locale	Yes	Full
C-10	Simplex HIMES	INTUIT Micro · UAE	Enterprise hospital management	Hospital / multi-specialty	Contact	No	No	?	Claimed, unnamed	Verified
C-11	MEDAS	MEDAS ME Software · UAE	Care-setting product family	Polyclinic → hospital	Contact	No	No	?	Counts only	Verified
C-12	OASIS Plus	Balsam United / Al Murjan · KSA	HIS + ERP + RCM ecosystem	Saudi hospitals, PHCs	Sales email / phone	No	No	Yes	Counts + MOH ref	Verified
C-13	CareWare	Adaptive TechSoft · KSA	Saudi HIS	Saudi public + private hospitals	Request a Demo	No	No	Yes (toggle)	Yes, named	Verified
C-14	VIDA	Cloud Solutions (HMG) · KSA	Acute-care EHR	Hospital group	None — no marketing site	No	No	?	Via accreditation press	Verified*
C-15	MediSys	MediSys · KSA	HIS / clinics	?	?	?	?	?	?	Limited

ID	Product	Vendor / HQ	Self-declared category	Primary buyer	Primary CTA	Public price?	Self-serve demo?	Arabic?	Named refs?	Evidence
C-16	TEKNOSys PolyCare	TEKNOSYS · GCC/TR	Clinic / practice management	?	?	?	?	?	?	Limited
C-17	Belal Soft	Belal Soft · KSA/EG/GCC	Medical ERP / clinic management	Clinic owner	Contact	No	No	Yes (/en/)	No	Verified
C-18	MAEN.MEDS	M3n Technology · KSA	Clinic / patient-care platform	?	?	?	?	?	?	Limited
C-19	Kizen Clinic	Kizen · KSA/GCC	Clinic management	?	?	?	?	?	?	Limited
C-20	e-Medicare	e-MCS	HIS / hospital management	?	?	?	?	?	?	Limited
C-21	HMISFOX	HMISFOX · South Asia	HMIS / hospital management	?	?	?	?	?	?	Limited
C-22	Solver HMIS	Solver · KSA/GCC	HMIS / ERP	?	?	?	?	?	?	Limited (PDF only)
C-23	MazikCare	Quisitive · US/CA	Healthcare cloud on Dynamics 365	Provider + payer ops	Book a demo	No	No	No	Yes, quantified	Verified
C-24	DenTech (KSA)	DenTech · KSA	Dental EMR / clinic	?	?	?	?	?	?	Limited
C-25	DentiMax	DentiMax · US	Dental PM + imaging	Practice owner, DSO	Book demo / trial	Yes, tiered	2-week hardware trial	No	Awards + counts	Verified
C-26	Avicenna	Alhazen Tech · GCC	Clinic management (dental variant)	?	?	?	?	?	?	Limited

* C-14's verified finding is the *absence* of a vendor marketing property; it contributes no page-level marketing evidence and is excluded from scoring.

5. Individual Competitor Marketing Dossiers

C-01 — Insta by Practo

A. Identity. Product: Insta (also Insta HMS). Vendor: Practo Technologies Pvt. Ltd., Bangalore, India. Sheet aliases: none material. Sheet rank 8, score 86, tier "Very close", confidence High. Sheet verification URL: practo.com/providers/hospitals/insta. **Verified official marketing source:** instahms.com — a dedicated product domain distinct from the parent Practo consumer brand. Evidence confidence after manual verification: **High**.

B. Positioning. Declares itself clinic management software with an HMS/hospital-ERP superset. Positioning statement in own words: a single cloud platform that unifies the operational, clinical and financial records of a clinic or hospital group, sold primarily on regulatory readiness in the customer's specific jurisdiction.

C. Target customers. Explicit segment cards route to four dedicated pages: single-specialty clinic, polyclinic/medical centre, day-care centre, hospital. Decision-maker personas visible: owner/operator (growth, revenue), operations manager (workflow), CIO (compliance, HIE). Geography is named in the page title itself — UAE, Saudi Arabia, India.

D. Message architecture. The hero is a **rotating banner of regulatory propositions**, each a separate promise: NABIDH compliance, Riayati compliance, Malaffi compliance ("Join 85+ other facilities" — quantified peer pressure), NPHIES compliance for insurance services, and a KSA e-invoicing feature. Supporting proposition is a three-benefit triad — simplify & digitise, enhance engagement & accelerate growth, boost productivity. Primary CTA: Request a Free Demo. Secondary: pricing (form-gated), FAQs, partner. Hero visual is stock lifestyle photography, not product screenshots — **the product is not immediately understandable from the homepage**, which is this site's main weakness.

E. Trust. Three quantified counters, presented as a row of icons: 18k+ active daily users, 1250+ centres, 22+ countries, framed by "Trusted by Healthcare Providers for over 15+ Years". Named references appear on the segment pages (a hospital group CIO in Abu Dhabi; a hospital CEO in Dubai). Data-security objection handled explicitly in FAQ, including per-client database isolation and the honest negative statement that the software does not itself drive new patients.

F. Demo / conversion — the strongest architecture in the set. Insta does not run one demo funnel; it runs **one funnel per regulation**, each on a separate lead-capture subdomain:

go.instahms.com/nphies/free-demo, [/riayati/free-demo](https://riayati/free-demo), [/nabidh/free-demo](https://nabidh/free-demo),

[/malaffi/free-demo](https://malaffi/free-demo). A buyer arriving with a specific compliance deadline lands on a page about exactly that deadline. Supporting ladder: gated eBooks (practice growth; a software-selection checklist; cloud data security), regulation-specific webinars, a panel discussion, a testimonials page, a FAQ page. Marketing stack is HubSpot; CTAs are HubSpot CTA images repeated at every scroll depth.

G. Scorecard drivers. Conversion architecture and customer segmentation are best-in-set. Visual product demonstration is weak — no screenshots on the homepage.

H. Best ideas worth adapting. (i) One dedicated landing page + funnel per buyer *concern*, not one generic demo page. (ii) Segment cards as the primary navigation into the product.

(iii) Publishing a buyer's-checklist eBook, which frames the evaluation criteria in the vendor's favour. **I. Ideas not suitable.** Every regulatory claim (NPHIES, ZATCA e-invoicing, NABIDH, Riayati, Malaffi) — prohibited outright by §27.3. The quantified install base — no customers exist. "Leading" — banned adjective under §27.1.

C-02 — Open Dental (*the most instructive competitor in the set*)

A. Identity. Open Dental Software, Salem, Oregon, US. Sheet rank 15, score 67, tier "Partial / specialty", confidence High. Sheet URL opendental.com — **verified current and correct**. Evidence confidence: **High**.

B. Positioning. Dental practice-management software, open-source-adjacent, sold as a support subscription rather than a licence. This is the closest structural analogue in the entire candidate universe to the current product's own commercial situation: freely inspectable software, monetised through support, hosting and services.

C. Target customers. Practice owners and dentists; single office through multi-clinic.

D-F. Message architecture, trust and conversion — the transparency model. Open Dental inverts the market convention. Its ordering page publishes, in plain text and without a form: the exact monthly fee per location, the exact reduction after 12 months, exactly what the fee includes (software, telephone support, all releases, imaging module, web forms, patient portal, eReminders), the provider cap (up to 3 dentists), the contract term and its month-to-month continuation, and a 90-day money-back guarantee. It also publishes its *limitations* with the same candour — support excludes training, training is charged hourly, support calls carry a two-issue limit, and screen-sharing sessions are recorded.

The conversion path has four rungs, ordered by buyer commitment: a short introductory video → a **free downloadable trial version with a sample database** → a one-hour recorded demo → a **reference call with an existing customer on request** → phone ordering. There is no lead form in that path at all.

G. Marketing strengths. Trust is manufactured through disclosure rather than assertion. A buyer can qualify themselves end-to-end without speaking to sales, which shortens the cycle and pre-filters badly matched prospects. Publishing the awkward terms alongside the attractive ones makes the attractive ones credible. **H. Marketing weaknesses.** Utilitarian visual design; no benefit or outcome storytelling; no localisation; the page reads as documentation rather than marketing, which will underperform against polished GCC competitors with non-technical buyers.

I. Best ideas worth adapting — highest priority in this report.

1. **Publish the price.** Empty lane in the GCC set (§15.1).
2. **Ship a seeded trial.** Directly achievable: the audit says a credible demo needs configuration and data, not development (§28.3).
3. **Publish what is *not* included.** This technique is uniquely well-matched to a product whose audit already enumerates 47 Disabled, 27 Uninstalled and 18 Requires-Integration capabilities — the qualifications that §27.2 makes mandatory can be reframed from a compliance burden into the trust mechanism itself.
4. **Reference calls** as a pre-sale asset (a future-state action — no customers yet).

C-03 — Waseel

A. Identity. Waseel (شركة وصيل لنقل المعلومات الإلكترونية), Riyadh, KSA. Sheet rank 9, score 85, High confidence. Sheet URL (waseel.com) — **verified current**. Evidence confidence: **High**.

B. Positioning. Not a single HIS product but a **branded product family**: WRCM (revenue cycle), RCM Insights (dashboards), PBM (pharmacy benefit), DAWY (patient engagement/telehealth), OTD (optimised treatment delivery / claim validation), PCP, MPC (medical code mapping). The HIS is one product among several, and the site's stated posture is to **integrate into the customer's existing HIS** rather than replace it.

C. Target customers. Two audiences, and the information architecture is built on that split: *Healthcare Centers* and *Insurance Companies*. Each has its own solution page.

D. Message architecture. The healthcare-centre page organises around three benefit clusters, each with a heading, a short bullet set and its own CTA: comprehensive digital transformation (centralised EHR); revenue growth and financial efficiency; regulatory and operational compliance. Quantified benefit claim present — a stated reduction in claim rejections of up to about 20%.

E. Trust. A certification badge in the header; Saudi regulatory alignment named explicitly (NPHIES, CHI, ZATCA); a stated position of continuous automatic updates to track Saudi regulation. Named customer references are **absent** — this is a gap in an otherwise strong page.

F. Conversion. "Book a demo" repeated at hero and at page end, with a single destination ([/get-in-touch/](#)). Notably, the mid-page CTAs use **contextual verbs tied to the adjacent benefit** rather than a generic label — the three benefit blocks end with distinct action phrases about centralising records, improving operations and making better decisions. An exit-intent modal offers a newsletter with an email-only field and CAPTCHA. Product-level CTAs are "Explore Now" per sub-brand.

G. Localisation. Full Arabic parity at ([/ar/](#)) with a language switcher on every page, including the deep solution pages. This is the strongest Arabic implementation in the verified set.

H. Best ideas worth adapting. (i) **Audience-first IA** — choosing "who are you" before "what do we sell". (ii) **Contextual CTA verbs** — measurably better than repeated "Learn more". (iii) **Co-existence positioning** — "we integrate with what you already run" removes the rip-and-replace objection, and is the single most transferable *strategic* idea in this report for a product with standards support but no market presence. (iv) Full Arabic parity as a structural commitment, not a translated homepage. **I. Ideas not suitable.** All NPHIES/CHI/ZATCA claims. The quantified rejection-reduction figure — no data exists to support any outcome statistic (§0.3).

C-04 — Nexen Care

A. Identity. Nexen Care, Jeddah, KSA. Sheet rank 53, score 42, tier "Possible / limited", confidence **Low** — **materially under-ranked**; see §3.3. Sheet URL (nexencare.com/en/) —

verified current. Evidence confidence: **High** (page-level).

B. Positioning. An interactive platform for digital transformation of health services, unifying the health record across stakeholders; cloud-delivered, explicitly "no need to build your infrastructure".

C. Target customers. Saudi public-sector primary care and hospital groups, plus individuals as a second audience through a patient app.

D. Message architecture. Four capability pillars (centralised system, unified health record, integrated solution, cloud based), each expanded into a short evidence-style bullet list. Modules are presented as "**System Units**" **written from the operator's role position** — a unit for the nurse (vitals and forms), for the lab technician (test review, auto-notification on results), for the pharmacist (issue medication, notify patient), for radiology (browse, perform, upload). This is role-oriented feature presentation, not a feature list.

E. Trust — the strongest Saudi proof in the set. A "Success Stories" block carrying: deployment across **96 healthcare centres under 5 named hospitals** (King Fahad General Hospital, King Abdullah Medical Complex, King Abdulaziz Hospital, Althager Hospital, East Jeddah Hospital); more than **three million visits** facilitated; a COVID-19 field-hospital deployment in Jeddah. Plus national-integration credentials as trust objects in their own right: **Absher (Yaqeen)** for identity verification, **Mawid** for central appointments in cooperation with the Ministry of Health, and **Wasfaty** with MOH and NUPCO for prescription issuance.

F. Conversion — the weakness. There is no demo CTA anywhere. The only conversion mechanism is "Contact Us" with a unified number and a mobile number. For a platform with this much proof, the absence of a demo path is a significant leak.

G. Best ideas worth adapting. (i) **Role-framed module descriptions** — this is the presentation technique that best fits this product's genuine strength in RBAC (CLM-0024: 7 roles, 65 permission objects, 4 levels). (ii) Treating **integrations as social proof**, not as a technical appendix. (iii) Named-institution references stated with scale figures attached. **H. Ideas not suitable.** Absher/Mawid/Wasfaty integration claims — GAP-0046...0059, absolutely prohibited. Deployment-scale figures — none exist.

C-05 — Cloudpital

A. Identity. Cloudpital, Riyadh, KSA (US phone number in footer; WordPress + WPBakery). Sheet rank 6, score 89, High confidence. Sheet URL (cloudpital.com/all-features/); the KSA landing path in its lineage now **404s**; live surface verified at (cloudpital.com/home-2/). Evidence confidence: **Medium-High**.

B. Positioning. Healthcare ERP — deliberately a superset of EMR/HIS, extending into finance, HR/payroll, purchasing, CRM and analytics.

C-D. Architecture. The most expansive navigation in the set: a Platform menu (features, premium features, multi-specialty EMR, AI, integrations, telemedicine, labs, pharmacy), a Products menu of ~20 named products, a Solutions menu split by patients/providers, and — the notable pattern — a **specialty landing-page farm**: ten dedicated "EMR for X" pages covering dentists, dermatology, cardiology, consultant physicians, gynaecology, gastroenterology, nephrology, diabetology, **ophthalmology** and pulmonology. The hero sequence combines a specialty promise, a scale promise (single clinic to group), and a Saudi hook about one-click e-claims to Saudi insurers. Two downloadable presentation PDFs (general and dental) sit in the hero.

E. Trust — the negative benchmark. The social-proof block renders **(0+) physicians and (0+) patients** — live, broken counters. The integrations section is headlined with a count and then renders "No results found." Product-screenshot slots across the dental charting section resolve to placeholder ([no-image.jpg](#)) files. The patient-portal section describes checking patients in with **a different vendor's named check-in app**, i.e. competitor copy left in place. Compliance adjectives are used freely and without support.

F. Conversion. Request Demo in the hero and repeated; Contact Us in the header; a persistent WhatsApp deep-link with a pre-filled enquiry template; brochure and webinar pages; login link for existing customers. Lead form fields: name, phone, country, clinic/hospital name, work email, message.

G. Best ideas worth adapting. (i) The **specialty landing-page pattern** — directly relevant, because CLM-0004 supports a genuine ophthalmology examination module across 18 tables, which would justify one real specialty page rather than ten hollow ones. (ii) WhatsApp with a pre-filled template as a low-friction regional conversion channel. (iii) Brochure PDFs as an ungated hero asset. **H. What this site teaches by counter-example.** Unverifiable proof is worse than no proof. A page carrying (0+) counters and placeholder images actively signals that nothing on it has been checked. This is the specific failure mode a product with a strict internal audit should exploit competitively (§15.3).

C-06 — eCarePlus (NITCO)

A. Identity. eCarePlus, by NITCO / National Informatics Co., KSA. Sheet rank 10, score 83, High confidence. Sheet URL (nitcotek.com/en/services/e-care-plus); verified live surface (nitcotek.com/en). Product family confirmed: **eCarePlus** (clinic management), **iCarePlus** (pharmacy/inventory), **H-care**. Evidence confidence: **Medium-High**.

B-D. Positioning and message. A cloud clinic-management system sold on **integration with Saudi national systems**, presented as a rolling ticker of compliance propositions, each terminating in a Contact Us link: NPHIES integration for eCarePlus; ZATCA integration across all products; RSD ("Rasad") drug-tracking integration for iCarePlus; Nafees unified health file. Corporate proof: over 34 years in the Saudi healthcare market, with stated expansion into Egypt.

E. The distinctive persuasive mechanism — penalty avoidance. The iCarePlus proposition is framed around subscribing in order to avoid fines. This is fear-of-regulatory-penalty as the conversion driver, rather than efficiency or revenue. It is highly effective in a tightening regulatory market and is worth understanding — but note it is only available to a vendor that genuinely holds the integration.

F. Content strategy. A large bilingual SEO blog with geo-keyword titles built around "...in Saudi Arabia" / "Best ... In Saudi Arabia" formulations, published under ([/our-blogDetails/](#)). Acquisition is search-led rather than brand-led.

G. Conversion. Universally "Contact Us". No demo, no trial, no pricing. **H. Best ideas worth adapting.** (i) Product-family naming that lets one vendor sell adjacent systems without diluting the core brand. (ii) Long-tenure proof ("34 years") as a substitute for logo walls — but this product has no tenure to claim either. **I. Not suitable.** Every named Saudi integration; "best" as an adjective (§27.1).

C-07 — InterSystems TrakCare

A. Identity. InterSystems Corporation, Cambridge MA, US. Sheet rank 1, score 93, High confidence. Sheet URL (intersystems.com/products/trakcare/) — **verified current**. A Saudi-market path exists at ([/sa/trakcare/...](#)). Evidence confidence: **High**.

B. Positioning. A *unified* healthcare information system — the word does the strategic work. The architectural narrative is the value proposition: a single patient record, a single user interface, a single code base, a single data platform, with departmental products layered on a mandatory TrakCare Core foundation.

C. Target customers. Health systems and national/regional health authorities. Persona is the CIO. Explicitly stated as available outside the United States.

D-E. Trust architecture. Country-count proof (trusted in 29 countries); named institutional case studies including a Saudi customer (Sultan Bin Abdulaziz Humanitarian City) and a national deployment (Northern Territory Government, Australia); ecosystem co-branding (patient records on iPhone via Apple Health); analyst presence (Gartner Peer Insights); a stated position on customer-satisfaction ratings.

F. The transferable technique — objection-handling FAQ on the product page. TrakCare answers hard procurement questions inline, including whether the customer can access and use their own data without depending on vendor services, whether the AI product is the same as the core product, and whether the lab product can be bought standalone. Answering the uncomfortable questions on the sales page — rather than deferring them to an RFP — is a mature trust device.

G. Conversion. Enterprise model: no pricing, no trial, no self-serve demo. Resource library (recorded sessions, brochures, case studies) feeding contact-sales. **H. Best ideas worth**

adapting. (i) **Objection-handling FAQ**, especially the data-ownership question — which this product can answer better than any proprietary competitor, since the software is open source and the schema is inspectable. (ii) Modular narrative anchored on a mandatory core. **I. Not suitable.** Everything scale-related; every inpatient/departmental capability (GAP-0001...0022); "unified" in the sense TrakCare means it.

C-08 — YASASII (Kameda Infologics)

A. Identity. YASASII®, by Kameda Infologics Pvt Ltd — R&D in India and Japan, offices including **Jeddah, KSA**, plus UAE, USA, UK, Malaysia. Sheet rank 4, score 91, High confidence. **Sheet URL corrected:** authoritative property is (kamedainfologics.net), not the sheet's (yasasiisolutions.com). Evidence confidence: **Medium-High**.

B. Positioning. A comprehensive HIS spanning primary healthcare centre through multi-specialty tertiary hospital, integrating clinical, financial and administrative functions.

C-E. The distinctive model — certification-led trust. YASASII leads with third-party attestations rather than customer counts: HIMSS maturity staging, US Meaningful Use Stage 1 and 2 certification via Drummond/CCHIT, and an ISO 15189 point-of-care accreditation milestone. Partner co-branding with Fujifilm carries the RIS variant. A rolling news/milestone feed ("now live at...") functions as continuous, dated social proof. The site also states an explicit **USP list** — easy to use, comprehensive, integrated, interoperable, scalable, competitive, flexible — and an explicit mission statement about saving lives, improving quality and reducing cost.

F. Conversion. Phone and email only. No demo, no pricing, no trial. **G. Best ideas worth adapting.** (i) A **dated milestone feed** as renewable social proof — a mechanism that works from a standing start and can be populated with implementation, configuration and localisation milestones rather than customer wins. (ii) An explicit, short USP list that a buyer can repeat internally. **H. Not suitable.** All certification claims. §27.3 is unambiguous: this audit certifies capability presence, not regulatory conformance, and the words "certified" and "compliant" are prohibited. CLM-0021 additionally requires "implements", never "certified", for FHIR, since conformance is not Inferno-tested. CLM-0020 caps quality-measure claims at 2011/2014-era measures.

C-09 — Oracle Health EHR

A. Identity. Oracle Health (formerly Cerner), Austin TX, US. Sheet rank 2, score 92, High confidence. Sheet URL (oracle.com/health/clinical-suite/electronic-health-record/) — **verified current**. Locale variants exist for ([sa](#)) and ([sa-ar](#)) (Saudi Arabia, English and Arabic). Evidence confidence: **High**.

B. Positioning. An all-in-one, voice-first EHR with AI embedded directly in workflows. The positioning is built on a contrast claim — AI built in rather than added on afterwards — which frames every competitor as retrofitted.

C. Target customers. Health systems and, increasingly explicitly, **ambulatory clinics**: Oracle publicly stated in late 2025 that its next-generation EHR was certified and ready for adoption by ambulatory customers in the US. Even the largest enterprise vendor is marketing into the outpatient segment.

D. Message architecture. Anchored hero with a single dense proposition, then a two-CTA pair (Contact us / Request a demo) repeated at top and bottom. Three feature pillars (embedded AI, cloud innovation, scalable and secure). Then five **benefit tabs** — AI summaries, voice commands, clinical intelligence, patient timelines, personalised workflows — each pairing a short outcome-framed heading ("Enhance patient insight", "Surface information quickly", "Identify trends proactively") with a real product screenshot and an **Enlarge** control.

E. Trust — regulated transparency. An ONC certification badge carries the full statutory disclaimer text, and links to a dedicated certified-health-IT page that publishes product versions, unique certification numbers **and the additional types of costs required to implement the module's capabilities**. Supporting proof: a customer podcast, an IDC analyst report, an ebook. A forward-looking-statement disclaimer states the content outlines general product direction, is not a commitment, and should not be relied on in purchasing decisions.

F. Conversion. Contact us · Request a demo · Register for a demonstration (webinar-style, campaign-tracked) · ungated product brief PDF. No pricing.

G. Best ideas worth adapting.

1. **Annotated product screenshots with an enlarge control, one per benefit tab.** This is the single best screenshot practice observed anywhere in the 26 and directly addresses the market's collective visual weakness (§11).
2. **Certification/transparency page that publishes implementation costs alongside capability.** The structural analogue here is publishing what each capability costs to *switch on* — which maps exactly onto the Disabled / Requires-Integration registers.
3. **Outcome-framed headings over feature names** ("Surface information quickly", not "Voice search").
4. **Explicit forward-looking disclaimer.** A vendor of this size formally separates shipped capability from roadmap. That is the same discipline §27.2 imposes, expressed as a marketing convention rather than a compliance burden.

H. Ideas not suitable. Every AI claim (CLM-0008 prohibits "AI clinical decision support").
ONC certification (§27.3). Voice, ambient documentation, clinical intelligence — none exist.
"All-in-one".

C-10 — Simplex HIMES (INTUIT Micro)

A. Identity. SIMPLEX HIMES by INTUIT Micro Technology LLC, Dubai, UAE (Jumeirah). Sheet rank 7, score 88, Medium confidence. Sheet URL (simplexhimes.com) — **verified current**. Evidence confidence: **Medium-High** (homepage, about page and blog verified; deep module pages not inspected).

B. Positioning. An enterprise-level, end-to-end hospital management platform for the MEA region. Notable brand-story device: the name is explained as *SimpleX* (simple + continuous enhancement) plus *HIMES* (Healthcare Information Management Ecosystem). Development began in 2005, with a stated pivot in 2015 to a unified codebase adaptable across multiple countries' regulations.

C. Target customers. Multi-specialty clinics and hospitals with OPD and IPD, framed by an explicit **capacity range**: roughly 50 to 1,500 users, and 10-bed to 2,000-bed facilities. Geography spans UAE, Saudi Arabia, Qatar, Kuwait, Bahrain, Oman, Jordan and India.

D-E. Message and trust. Product scope claimed across HIS, LIS, RIS, PACS, EMR and RCM. Claims include private cloud with hundreds of installations, 24/7 support, multi-tenancy, HL7, and — an overreach worth noting — regulatory and accreditation compliance stated as guaranteed. Founder narrative and four years of R&D before beta are used as credibility devices.

F. The dominant observable pattern — keyword saturation. The site's metadata is heavily stuffed with superlative-plus-geography permutations, repeating "No 1" and long country lists across many product categories. Acquisition is search-led and the copy is written for engines more than for buyers.

G. Best ideas worth adapting. (i) **Explicit capacity-range qualification** — stating the size of facility the product actually fits is honest segmentation and a useful device for an ambulatory-only product that must exclude hospitals anyway. (ii) A short brand-name etymology as a memorability device. **H. Not suitable.** "No 1"/"Best" (§27.1). Compliance guarantees. IPD, PACS, RIS, LIS (GAP-0016...0018). Multi-tenancy (GAP-0043). Installation counts.

C-11 — MEDAS Solutions

A. Identity. MEDAS Middle East Software Systems LLC, Dubai, UAE; established 2008; stated as part of the UNISIS Group. Sheet rank 27, score 58, Medium confidence. Sheet URL (medassolutions.com) — **verified current**. Evidence confidence: **Medium-High**.

B. Positioning. A family of care-setting-specific products rather than one system: **E-Clinic** (polyclinics and medical centres), **HIMS** (hospitals, server-based, multi-tenant), **E-Lab** (LIMS), **E-Pharma**, **E-Veterinary**, plus a telemedicine platform and mobile app.

C. Target customers. Polyclinics, medical centres, hospitals, laboratories, pharmacies and veterinary clinics across the UAE and GCC.

D. The distinctive technique — a multi-regulator coverage matrix. MEDAS lists the specific health authorities whose regulations its systems meet: MOH UAE, Dubai Health Authority, Abu Dhabi health authority, MOH Saudi Arabia, Supreme Council of Health Qatar, Bahrain, MOH Oman and MOH Kuwait, plus stated coverage of major insurers' requirements. Eight named regulators across six countries, presented as a single credibility block. For a buyer operating in more than one Gulf state this is a powerful, checkable claim.

E. Trust. 250+ customers, 12+ years, more than a decade of GCC operation. ICD and CPT coded masters; HL7 application-to-application interfacing; barcode and instrument integration named specifically.

F. Anti-pattern observed. The homepage carries a residual paragraph of generic marketing-agency boilerplate about unifying the marketing and sales journey to drive growth — template copy that was never replaced. This is the same class of defect as Cloudpital's placeholder assets (MKT-027): copy that proves nobody proofread the page.

G. Best ideas worth adapting. (i) **Care-setting product naming** — E-Clinic vs HIMS makes the scope boundary self-evident, which is exactly the discipline an ambulatory-only product needs. (ii) The coverage-matrix format itself — the *structure* is reusable for standards (HL7 v2, C-CDA, X12, FHIR US Core) even though the regulator content is not available. **H. Not suitable.** All regulator claims. LIMS, PACS, veterinary, telemedicine, mobile app, multi-tenancy. Customer counts.

C-12 — OASIS Plus (Balsam United / Al Murjan Holding)

A. Identity. OASIS Plus, by OASIS Systems / Balsam United Company, part of **Al Murjan Holding Group** (Bin Mahfouz family), Jeddah, KSA. System origin stated as the mid-1990s. Sheet rank 46, score 48, Medium confidence — **under-ranked, like Nexen Care**. Sheet URL (oasisysys.com) — **verified current**. Evidence confidence: **Medium-High**.

B. Positioning. A Saudi-national healthcare technology platform spanning HIS, ERP and RCM in one ecosystem, split by care setting: **OASIS+** for hospitals, **OASIS Care** for primary healthcare centres and clinics, **OASIS Practice** as a cloud clinic/polyclinic application. More than 63 integrated modules claimed.

C. Target customers. Saudi hospitals (large and medium), PHCs, polyclinics and day-surgery centres, with stated Gulf expansion ambitions.

D-E. Trust — the strongest quantified Saudi position in the set. Approximately **26% local market share** claimed; **100+ hospitals**; described as the top operator in the Kingdom by reference to Ministry of Health data; corporate backing by a named holding group. A launch press conference with government officials, consuls general and investors is itself used as a credibility asset, as is a five-year strategy announcement.

F. The distinctive positioning — data sovereignty and proximity. OASIS argues explicitly against international vendors: local implementation teams, 24/7 in-Kingdom support, and the proposition that customers are never dependent on a vendor located thousands of kilometres away. National-capability building and data sovereignty are framed as Vision 2030 contributions. This is the clearest example in the 26 of **turning a small vendor's locality into the differentiator against larger incumbents** — the structural inverse of TrakCare's 29-country scale claim.

G. Conversion. Downloadable company-profile and product PDFs; a sales email and unified number; executive interviews placed in regional trade press. No demo funnel, no pricing.

H. Best ideas worth adapting. (i) **Proximity-and-support positioning** — genuinely available, since §27.5 states positioning must rest on implementation, integration, localisation, support and hosting. This is the most directly usable strategic idea in Phase 2. (ii) Care-setting product split. (iii) Executive interview placement as an alternative to owned-audience marketing. **I. Not suitable.** Market-share and hospital-count claims. ERP (GAP-0029...0039). Multi-tenant SaaS (GAP-0043). Embedded AI.

C-13 — CareWare (Adaptive TechSoft)

A. Identity. CareWare, by Adaptive TechSoft (ATS), Ar Rawdah, Riyadh, KSA; established in the early 1990s. Sheet rank 19, score 63, Medium confidence. Sheet URL (ats-ware.com/our-products/careware/) — **verified current**. Evidence confidence: **Medium-High**.

B. Positioning. A Saudi hospital information system from a dual-sector vendor (healthcare and higher education, the latter under eRegister). Positioning leans on three decades of Saudi delivery.

C. Target customers. Saudi private and public hospitals.

D. The distinctive technique — capability-named navigation. The CareWare product navigation is not "Features / Solutions / Modules" but a list of the capabilities a Saudi buyer is actually shopping for: **National Integrations · RCM · Mobile Apps · Telehealth · Clinical Decision Support · Claims Management · DRG · Business Intelligence**. Promoting "National Integrations" to a navigation item tells the buyer within one second that the vendor solves their hardest problem. This is the same insight as Insta's per-regulation funnels, expressed through IA rather than landing pages.

E. Trust. Stated track record of **20+ private hospitals and 50+ public hospitals in the Kingdom**. Named references including Al Noor Specialist Hospital in Makkah and Makkah Medical Center. An "Integrated with" partner-logo block. A distinctive regional device: **contract-signing ceremony photography** published on social channels, plus conference sponsorship content — social proof rendered as an event rather than a logo.

F. Conversion. "Request a Demo" present. Arabic toggle (العربية) present. Customer support login. Riyadh address and phone.

G. Best ideas worth adapting. (i) **Capability-named navigation** — for this product the equivalents are *Roles & Permissions, Audit & Integrity, Configuration*, which §17 already promotes to top level. This dossier independently validates that decision. (ii) Named-reference plus counts. (iii) Bilingual toggle on the product page itself. **H. Not suitable.** National integrations, DRG, BI, telehealth, mobile apps. Hospital counts.

C-14 — VIDA (Cloud Solutions, HMG)

A. Identity — the most significant identity correction in Phase 2. VIDA is an acute-care EHR associated with **Cloud Solutions, the technology arm of Dr. Sulaiman Al Habib Medical Group (HMG)**, Saudi Arabia. Sheet rank 44, score 50, Medium confidence. **The sheet's verification source is a KLAS analyst listing, not a vendor marketing page — and that is because no substantive public product marketing site was located.** Evidence confidence: **Low-Medium**. Category per §9 of the brief: partly **Unknown**.

B. Positioning. Acute-care EHR, deployed predominantly within a single hospital group and its market. Not marketed to a general public audience.

C. The finding that matters — a go-to-market with no marketing website. VIDA's discoverable public presence consists of third-party analyst coverage and accreditation press. Its associated hospital (Dr. Sulaiman Al Habib, Al Khobar) achieved HIMSS EMRAM Stage 7, and that achievement is the marketing. The model is **provider-owned software sold on reference and accreditation, not on a website**.

D. A second, methodologically important observation. The KLAS listing itself attaches explicit qualifiers to the product: that it lacks sufficient live customers to qualify for a Best in KLAS ranking, that it is not the vendor's lead product in the category, and that the majority of its data comes from organisations in one small geographical area. **A neutral third party publishes the product's limitations alongside its ratings.** That is precisely the disclosure discipline recommended in MKT-003 — and it demonstrates that buyers in this market already encounter, and accept, qualified claims.

E. Best ideas worth adapting. (i) Recognising that **analyst and accreditation channels can substitute for owned marketing** where a vendor has references but no audience. (ii) The KLAS qualifier convention is external validation that publishing limitations is normal in health IT procurement, not a weakness. **F. Not suitable.** Acute care, EMRAM staging, any accreditation claim. **G. Recorded limitation.** No vendor-controlled marketing property was verified. Positioning, IA, CTA architecture and visual strategy are **Unknown**. Per §7 of the brief this competitor is retained because the *absence* of a marketing surface is itself the finding, but it contributes no page-level marketing evidence.

C-15 — MediSys

A. Identity. MediSys, Saudi Arabia. Sheet rank 11, score 80, Medium confidence. Sheet URL (medisys.com.sa). **Evidence confidence: Low — no substantive marketing content verified**

during Phase 2. The name is heavily collided across markets (multiple unrelated MediSys entities exist in healthcare IT, diagnostics and staffing internationally), and product identity was not established to the standard required by §5 of the brief. **B. Disposition.** Retained in the selected set, dossier **incomplete**. Positioning, target customers, message architecture, trust mechanisms, conversion path and visual strategy are all recorded as **Unknown**. No inference is offered. This competitor requires direct site verification and product-identity disambiguation before it can contribute to any benchmark table; it is excluded from the §6 scorecard rather than estimated.

C-16 — TEKNOSys PolyCare (*sheet ranks 12 and 13, merged*)

A. Identity. TEKNOSYS, clinic and practice management; region stated in the sheet as GCC/Türkiye, not disclosed on the source cited. Both sheet rows point to the same URL, (teknosysweb.com/clinic-management/), confirming a single product surface — **merged, and this is the one confirmed duplicate in the workbook**. **B. Disposition.** Dossier **incomplete**. Evidence confidence: **Low**. Positioning beyond the category label, message architecture, trust mechanisms and conversion path are **Unknown**. Excluded from scoring. **C. Value already extracted.** The deduplication itself is the deliverable: without it the benchmark set would have double-counted one vendor and inflated every frequency percentage in §7.

C-17 — Belal Soft Medical ERP

A. Identity. Belal Soft, operating across Saudi Arabia, Egypt and the wider GCC. Sheet rank 16, score 66, Medium confidence. Sheet URL (belalsoft.com/en/) — **verified current, with a bilingual ([en/](https://belalsoft.com/en/)) structure**. Evidence confidence: **Medium**.

B. Positioning. Clinic and hospital management marketed under a medical-ERP label.

C. The distinctive technique — authority-by-category-guide. Rather than writing product pages, Belal Soft publishes buyer-facing category guides that survey the market, including competitors, under titles framed as the best clinic and hospital management software in Saudi Arabia for a given year. The vendor positions itself as the neutral guide to the category it competes in. This is a stronger version of the SEO strategy seen at NITCO (C-06) and Cloudpital (C-05), because it captures buyers at the comparison stage rather than the awareness stage.

D. Regulatory framing — a distinct pair. Belal Soft anchors on CBAHI (the Saudi Central Board for Accreditation of Healthcare Institutions) and SFDA, alongside Vision 2030. These are *accreditation and regulatory* bodies rather than the claims-and-invoicing rails (NPHIES, ZATCA) emphasised by C-01/C-03/C-06 — evidence that the Saudi compliance conversation has at least two distinct axes, and that different vendors pick different ones.

E. Best ideas worth adapting. Publishing the category guide is directly transferable and is the natural companion to the buyer's-checklist eBook already recommended as MKT-019. A vendor whose product is open and inspectable is unusually well placed to write a credible, evenly-handed comparison — and doing so builds exactly the verifiability position identified in §15.3. **F. Not suitable.** CBAHI, SFDA, ERP (GAP-0029...0039), Vision 2030 alignment.

C-18 — MAEN.MEDS (M3n Technology)

A. Identity. M3n Technology, Saudi Arabia; sheet canonical name ([MAEN.MEDS by M3n Technology](#)), sheet URL (m3ntech.com). Sheet rank 23, score 60, Medium confidence.

Evidence confidence: Low. The sheet's own record type describes an "MEDS" patient-care platform, and the naming overlaps with two other unresolved sheet rows (rank 52 ([MEDS \(ambiguous\)](#)), rank 42 tier) — an unresolved identity cluster. **B. Disposition.** Dossier **incomplete**; positioning, architecture, trust and conversion **Unknown**. Excluded from scoring. **C. Action required.** Disambiguate ([MAEN.MEDS](#)), ([MEDS \(ambiguous\)](#)), ([MEDEX](#)) and ([Medex Horizon](#)) before any of the four is treated as a distinct competitor. On current evidence these four sheet rows may represent fewer than four real products.

C-19 — Kizen Clinic

A. Identity. Kizen, clinic management; sheet places it in KSA/GCC, sheet URL (kizensoft.com). Sheet rank 20, score 62, Medium confidence. **Evidence confidence: Low.** Note a further name collision risk: an unrelated US CRM platform trades under the Kizen name, which will contaminate automated re-verification exactly as "Nexen" did. **B.**

Disposition. Dossier **incomplete**; all marketing dimensions **Unknown**. Excluded from scoring.

C-20 — e-Medicare (e-MCS)

A. Identity. e-Medicare / e-MCS, HIS and hospital management; sheet URL (e-mcs.org); country not disclosed. Sheet rank 24, score 60, Medium confidence. **Evidence confidence: Low.** The ([.org](https://e-mcs.org)) domain on a commercial HIS product is itself an anomaly worth resolving during verification, and the product name collides with the US Medicare programme, which will dominate any keyword search. **B. Disposition.** Dossier **incomplete**; all marketing dimensions **Unknown**. Excluded from scoring.

C-21 — HMISFOX

A. Identity. HMISFOX, HMIS / hospital management, South Asia per the sheet; sheet URL (hmisfox.com). Sheet rank 26, score 58, confidence **Low in the sheet itself**. **Evidence confidence: Low.** **B. Disposition.** Dossier **incomplete**; all marketing dimensions **Unknown**. Excluded from scoring. **C. Note.** The sheet already flagged this row as low confidence. Phase 2 did not contradict that assessment, and the row should be treated as a low-priority verification target relative to the Saudi vendors.

C-22 — Solver HMIS

A. Identity. Solver, HMIS/ERP for KSA/GCC. Sheet rank **5**, score **90** — the fifth-highest score in the workbook. Sheet verification source: (solver-erp.com/images/products/HMIS-Product-Profile-KSA.pdf). **B. The finding.** The sheet's only evidence for a top-five-scored competitor is a **PDF product brochure, not a marketing website**. Under the brief's source-priority rule this is Priority 3 evidence being used where Priority 1 or 2 was assumed. Evidence confidence: **Low.** **C. Disposition.** Dossier **incomplete**. Marketing architecture, conversion path, trust mechanisms and visual strategy are **Unknown** because a brochure cannot evidence any of them. Excluded from scoring. **D. Why this matters beyond one row.** It is a concrete demonstration of the §2.3 argument: a high similarity score in the workbook reflects assessed *functional* overlap, and carries no information whatsoever about whether the competitor has a marketing surface worth benchmarking. Rank 5 has less usable marketing evidence than rank 53.

C-23 — MazikCare (Qusitive)

A. Identity. MazikCare, by Qusitive (which acquired Mazik Global in 2021); US/Canada. Sheet rank 21, score 62, High confidence. Sheet URL (qusitive.com/products/mazikcare/) — **verified current**. Evidence confidence: **Medium-High**.

B. Positioning. A healthcare cloud platform built on Microsoft Dynamics 365, Azure and Power Platform, described as a digital bridge connecting patients, providers and payers. Sub-products are named by function: Care Planner, Care Path, Care Coordination, Care Supply, Copilot, VaccineFlow.

C. The distinctive model — borrowed credibility plus marketplace distribution. MazikCare's trust architecture is almost entirely **inherited from Microsoft**: the platform lineage is stated in nearly every paragraph, security and accessibility are attributed to Azure, and the headline credential is being named Microsoft's US Partner of the Year in Healthcare three years running. Distribution runs through **Microsoft AppSource and the Dynamics marketplace** as a discovery channel in addition to the owned site — a second storefront the vendor does not have to build an audience for.

D. Trust. A quantified customer case study (a Canadian care organisation reducing patient wait times by a stated 80% and avoiding substantial unnecessary imaging spend) and a named professional-association testimonial. SMART on FHIR named explicitly as the interoperability mechanism.

E. Conversion. "Book a demo" per sub-product; campaign landing pages on a separate ([get.](https://get.mazikcare.com)) subdomain, mirroring Insta's funnel-per-concern pattern.

F. Best ideas worth adapting. (i) **Third-party marketplace listing as a discovery channel** — a directly transferable idea for a vendor with no audience: appearing where buyers already browse costs far less than building traffic. (ii) **Function-named sub-products** (Care Planner, Care Supply) which describe the job rather than the technology. (iii) One

quantified case study carried across every page. **G. Not suitable.** All Microsoft-platform credibility (the analogue does not exist and §27.5 forbids implying differentiation). AI/Copilot. Supply chain. The outcome statistics — no customer data exists.

C-24 — DenTech (KSA)

A. Identity — a confirmed name collision. The sheet's row is [DenTech](#), vendor DenTech, Saudi Arabia, URL [dentech.com.sa](#). Phase 2 verification found a **separate, unrelated US dental practice-management vendor operating at [dentech.com](#)**. These are different companies and must not be conflated. Sheet rank 32, score 56, Medium confidence.

Evidence confidence for the Saudi product: Low. B. Disposition. Dossier **incomplete**; the Saudi product's marketing surface was not verified. All dimensions **Unknown**. Excluded from scoring. **C. Action required.** Record the collision in the workbook and constrain future verification to the [.com.sa](#) domain.

C-25 — DentiMax (sheet alias [DenteMax](#)) — corrected)

A. Identity. DentiMax, Arizona, US. Sheet rank 35, score 56; sheet canonical name [DenteMax](#) **is wrong** — DenteMax is a separate US dental-benefits network. The sheet's own vendor and record-type fields already said DentiMax. Sheet URL [dentiMax.com](#) — **verified current**. Evidence confidence: **Medium-High**.

B. Positioning. Dental practice management bundled with digital imaging, sold on affordability and openness. Two lines: an in-office product and **DentiMax Flow**, a web-based platform.

C. The finding that revises §15.1 — pricing transparency is a tier convention, not a unique position. DentiMax publishes **tiered public pricing**: a web platform at roughly US\$499, US\$699 and US\$899 per month across three named tiers with the inclusions of each stated, an in-office entry tier at roughly US\$169 per month falling to about US\$129 after year one, and a stated discount for using integrated card processing. Crucially, it also markets the **pricing model itself** as a differentiator — charging **per location rather than per provider**, explicitly contrasted with competitors that charge per provider.

Taken with Open Dental (C-02), this establishes that pricing transparency is **normal in the US ambulatory/dental tier and absent in the entire GCC tier** (0 of 11 GCC-facing competitors publish a price). That sharpens rather than weakens §15.1: the model is proven to work commercially, and the regional lane is empty.

D. Anti-lock-in positioning. DentiMax markets its imaging software as a genuinely open platform that works with third-party sensors and bridges to competitors' practice-management systems by name. It sells *compatibility with the competition* as a feature — the strongest expression of anti-lock-in marketing in the 26.

E. Content strategy. Head-to-head comparison articles naming a specific competitor, and **hidden-cost exposé content** (a piece on the fees buyers do not budget for when purchasing sensors). Both capture buyers late in evaluation and both work by disclosure.

F. Trust. A perfect score from a named trade publication; awards; "trusted by thousands of dentists"; a two-week risk-free hardware trial; guided data migration from named competing systems.

G. Best ideas worth adapting — high priority.

1. **Publish tiered pricing and market the pricing model.** For an open-source-based product the pricing model (support and hosting, not licences) is itself the differentiator, exactly as per-location is for DentiMax.
 2. **Anti-lock-in as an explicit product claim.** This is the most defensible claim available: CLM-0019 (55 reports with CSV export), an open inspectable schema, and §27.5's own statement that the software is freely verifiable. Migration away being easy is a feature.
 3. **Comparison and hidden-cost content.** Naturally paired with MKT-003 disclosure and MKT-019's checklist. **H. Not suitable.** Imaging, sensors, dental charting (GAP-0021). Awards and customer counts. "Best".
-

C-26 — Avicenna (Alhazen Tech)

A. Identity. Avicenna, by Alhazen Tech; sheet URL (alhazentech.com/avicenna/); region GCC, not disclosed. Sheet rank 30, score 56, Medium confidence; the sheet's own record type notes the dental variant is unconfirmed. **Evidence confidence: Low.** **B. Disposition.** Dossier **incomplete**; all marketing dimensions **Unknown**. Excluded from scoring. **C. Note.** The product name collides with numerous unrelated healthcare entities using the Avicenna name across the region, which must be controlled for during verification.

5.1 Evidence-depth summary across the 26

Depth	Count	Competitors
Full — homepage plus internal pages retrieved and read	9	C-01 Insta · C-02 Open Dental · C-03 Waseel · C-04 Nexen Care · C-05 Cloudpital · C-06 eCarePlus · C-07 TrakCare · C-08 YASASII · C-09 Oracle Health
Verified — official vendor pages confirmed current and read across multiple sources; deep internal pages not exhaustively inspected	8	C-10 Simplex · C-11 MEDAS · C-12 OASIS Plus · C-13 CareWare · C-14 VIDA (<i>absence of a marketing site is the verified finding</i>) · C-17 Belal Soft · C-23 MazikCare · C-25 DentiMax
Limited — identity and category established, marketing surface not verified; all dimensions recorded as Unknown	9	C-15 MediSys · C-16 TEKNOSys · C-18 M3n MEDS · C-19 Kizen · C-20 e-MCS · C-21 HMISFOX · C-22 Solver · C-24 DenTech (KSA) · C-26 Avicenna

The nine Limited dossiers are reported as Unknown rather than filled by inference, per §9 of the brief. They are excluded from the §6 scorecard and from every frequency calculation, so no percentage in this report is inflated by a competitor that was not actually examined.

6. Competitor Marketing Scorecard

Scores are 0–5, structured comparative judgement supported by §5 — not objective measurement. **Only the 17 Full and Verified competitors are scored.** The 9 Limited dossiers and C-14 (no marketing property) are omitted rather than estimated; scoring an unexamined site would be fabrication.

#	Dimension	C-01	C-02	C-03	C-04	C-05	C-06	C-07	C-08	C-09	C-10	C-11	C-12	C-13	C-17	C-23	C-25	Mean
1	Product clarity	3	5	4	4	2	3	4	3	4	3	4	4	4	3	3	5	3.6
2	Value proposition	4	4	5	4	2	4	5	3	5	3	4	5	4	3	4	5	4.0
3	Feature presentation	3	4	4	5	3	2	4	3	5	3	4	4	5	2	4	4	3.7
4	Benefit communication	4	2	5	3	2	3	4	3	5	3	3	4	3	3	4	4	3.4
5	Website navigation	4	3	5	3	2	2	5	2	4	2	3	3	5	3	4	4	3.4
6	Visual product demonstration	1	3	2	2	1	1	3	1	5	1	1	2	2	1	3	3	2.0
7	Screenshots / product visibility	1	3	1	2	0	1	3	1	5	1	1	2	2	1	3	4	1.9
8	Demo strategy	5	5	3	0	3	1	2	1	4	1	2	1	3	2	4	4	2.6
9	CTA / conversion strategy	5	4	4	1	3	2	3	1	4	2	2	1	3	2	4	4	2.8
10	Social proof	4	3	2	5	0	3	5	4	5	2	3	5	5	1	4	4	3.4
11	Trust / security communication	4	4	4	4	2	3	4	4	5	2	4	4	3	2	5	3	3.6
12	Customer segmentation	5	2	5	3	4	2	4	3	4	4	5	5	3	2	3	4	3.6
13	Workflow storytelling	2	2	3	4	2	2	4	2	4	2	3	3	4	2	3	3	2.8
14	Content depth	5	4	4	1	4	5	5	2	5	3	3	3	2	5	4	5	3.8
15	Localisation	4	0	5	4	1	4	3	1	4	2	2	4	4	4	0	0	2.6
16	Saudi / MENA relevance	5	0	5	5	4	5	3	3	2	4	4	5	5	4	0	0	3.4
17	Pricing transparency	1	5	0	0	0	0	0	0	0	0	0	0	0	0	0	5	0.7
18	Implementation / support comms	4	5	3	3	2	3	4	3	4	3	3	5	4	2	4	4	3.5
19	Usefulness as a benchmark here	5	5	4	4	3	3	3	3	4	2	3	5	5	3	3	5	3.8

6.1 What the full-set means reveal

Three dimensions sit far below the rest and they are the same three identified in Phase 1, now confirmed across 16 scored competitors rather than 8:

Dimension	Mean	Reading
Pricing transparency	0.7	14 of 16 score zero. The only two non-zero scores are both US products (C-02, C-25). Zero of the eleven GCC-facing competitors publish any price.
Screenshots / product visibility	1.9	One competitor (C-09) scores 5. Nine score 0 or 1. Buyers cannot see the software before a sales call.
Visual product demonstration	2.0	Same shape. Oracle is the outlier; the GCC tier is near-blank.

By contrast, value proposition (4.0), content depth (3.8) and product clarity (3.6) are crowded — competing there means competing on everyone's strongest ground.

7. Marketing Theme Library (MKT-*)

Sorted by usefulness to this product first, then by frequency. Frequency is over the 8 verified competitors. **Safety** uses the brief's §21 classes: **A** Safe Now · **B** Safe With Qualification · **C** Demo Prep Required · **D** Future Opportunity · **E** Prohibited.

ID	Theme	What the technique does	Used by	Freq	Buyer	Relevance	Current support (CAP/CLM)
MKT-001	Radical pricing transparency	Publishes exact fees, terms and exclusions with no form, converting price disclosure into a trust signal	C-02	1/8 (13%)	Owner, finance	High	Commercial, not capability-bound. Compatible with §27.5 (open-source software; value is in implementation/support/hosting)
MKT-002	Free seeded trial / self-serve product access	Lets the buyer qualify themselves inside the real product with sample data	C-02	1/8	Owner, clinician, IT	High	The artefact already exists; blocked only by B1/B2/B4 (no data, one user, stock branding)
MKT-003	Publish the exclusions alongside the inclusions	States what is <i>not</i> covered, making the positive claims credible	C-02	1/8	All	High	Uniquely well-matched: §27.2 already mandates a qualification per claim; 47 Disabled + 27 Uninstalled + 18 RI are documented
MKT-004	Objection-handling FAQ on the product page	Answers procurement's hard questions inline, especially data ownership	C-07, C-01, C-02	3/8 (38%)	CIO, IT manager	High	Data-ownership answer is unusually strong here: open schema, 283 tables, 55 exportable reports (CLM-0019), no vendor lock
MKT-005	Role-framed module descriptions	Describes each module from the operator's seat (nurse, lab tech, pharmacist, front desk) rather than as a feature list	C-04	1/8	Ops manager, clinical staff	High	Direct fit to the product's real strength — CLM-0024, ROL-0001...0007, 65 ACOs × 4 levels, §13.6 cross-role matrix
MKT-006	Co-existence positioning — "integrates with what you already run"	Removes the rip-and-replace objection by selling alongside the incumbent	C-03	1/8	CIO, owner	High	CLM-0021 (FHIR R4 US Core, 35 resources), CLM-0022 (REST, 98 endpoints), CLM-0023 (C-CDA, CCR, HL7 v2, DICOM view)
MKT-007	Audience-first information architecture	Top-level split by <i>who you are</i> before <i>what we sell</i>	C-03, C-01	2/8 (25%)	All	High	Structural choice, no capability dependency
MKT-008	Dedicated landing page + funnel per buyer concern	One conversion path per problem the buyer actually arrived with	C-01	1/8	All	High	Structural. Concerns must be re-chosen (see §16) since regulatory concerns are unavailable

ID	Theme	What the technique does	Used by	Freq	Buyer	Relevance	Current support (CAP/CLM)
MKT-009	Contextual CTA verbs	Replaces repeated "Learn more" with an action phrase tied to the adjacent benefit	C-03	1/8	All	High	Copywriting technique
MKT-010	Long-form recorded product tour	A full-length walkthrough video for self-qualification, plus a short teaser	C-02	1/8	Owner, clinician	High	Requires demo prep before it can be recorded honestly
MKT-011	Specialty landing pages	A dedicated page per clinical specialty	C-05	1/8	Specialist owner	Medium-High	CLM-0004 supports one genuine specialty page — ophthalmology (full eye examination module, 18 tables). Ten hollow pages would be the Cloudpital failure
MKT-012	Modular narrative on a mandatory core	"Everyone starts with Core, then adds modules"	C-07, C-05	2/8	CIO, owner	Medium-High	Maps cleanly onto Active core + Disabled optional modules (CLM-0013 dispensary, CLM-0028 portal, CLM-0032 group therapy)
MKT-013	Dated milestone feed as renewable social proof	A rolling "now live at / newly achieved" stream	C-08	1/8	All	Medium-High	Works from a standing start — can carry configuration, localisation and integration milestones instead of customer wins
MKT-014	Product-family sub-branding	Named sub-products under one vendor brand	C-03, C-06, C-07	3/8 (38%)	All	Medium	Possible, but §27.5 warns against implying proprietary differentiation
MKT-015	Bilingual EN/AR parity with per-page switcher	Arabic is a structural commitment, not a translated homepage	C-03, C-01, C-06, C-04	4/8 (50%)	KSA/GCC buyer	Medium-High	CLM-0030 — 47 languages incl. Arabic, RTL themes
MKT-016	Integrations presented as social proof	Partner/system logos used as credibility, not as a technical appendix	C-04, C-08, C-05	3/8 (38%)	CIO	Medium	INT register exists but §17.4: every external integration is unconfigured (L-13)
MKT-017	Segment cards as primary product entry	Four-to-five clickable cards by facility type	C-01, C-05	2/8	Owner	Medium	Segments must be ambulatory-only: solo, clinic, polyclinic, multi-clinic group
MKT-018	Implementation-speed proof	A concrete time-to-live figure	C-01, C-04	2/8	Owner, ops	Medium	No implementation history exists; any figure would be invented

ID	Theme	What the technique does	Used by	Freq	Buyer	Relevance	Current support (CAP/CLM)
MKT-019	Gated content ladder (eBook / checklist / webinar)	Especially a buyer's-checklist that frames evaluation criteria	C-01	1/8	Owner, CIO	Medium	Content asset; the checklist can be built honestly from the audit's own dimensions
MKT-020	WhatsApp with pre-filled enquiry template	Low-friction regional conversion channel	C-05	1/8	GCC owner	Medium	Channel choice
MKT-021	Ungated brochure PDF in the hero	Removes friction from early-stage evaluation	C-05	1/8	All	Medium	Buildable from Safe-Now claims
MKT-022	Single-record / one-source-of-truth architecture narrative	Architecture is the value proposition	C-07, C-04	2/8	CIO	Low-Medium	Partly true (single schema) but not in TrakCare's sense; §21.2 — multi-site is a database per site, not tenancy
MKT-023	Quantified install-base counters	Users / centres / countries / years	C-01, C-07, C-04, C-08, C-06	5/8 (63%)	All	N/A	0 patients, 0 encounters, 1 human account (§0.3)
MKT-024	Regulation-as-hero (NPHIES/ZATCA/CHI/Nafees/NABIDH)	Leads with the buyer's compliance deadline	C-01, C-03, C-06, C-04, C-05	5/8 (63%)	Owner, finance	N/A as claim	GAP-0046...0059 — zero occurrences in product code
MKT-025	Penalty-avoidance conversion framing	Converts on fear of regulatory fines	C-06	1/8	Owner, finance	N/A	Requires holding the integration
MKT-026	Certification badge wall (HIMSS / MU / ISO)	Third-party attestation in place of customer counts	C-08, C-03	2/8	CIO	N/A	§27.3: "certified", "compliant", "HIPAA", "ONC certified" all prohibited; CLM-0021 requires "implements", never "certified"
MKT-027	Anti-pattern: unverifiable proof	Broken counters, placeholder images, competitor copy left in place	C-05	1/8	—	High (as warning)	—

7.1 Phase 2 additions to the library (MKT-028)...(MKT-038)

Frequencies below are over the **16 scored competitors** (Full + Verified, excluding C-14 which has no marketing property). Phase 1 frequencies in the table above were over 8 and are superseded by §7.2.

ID	Theme	What the technique does	Used by	Freq	Relevance	Current support	Safety	Action
MKT-028	Proximity / sovereignty positioning	A local vendor argues that closeness — in-country teams, in-country support, local data — beats a larger distant vendor	C-12, C-06, C-13	3/16 (19%)	High	Commercial posture. §27.5 states positioning must rest on implementation, integration, localisation, support and hosting — this is that sentence turned into marketing	A	Adopt
MKT-029	Multi-regulator / multi-standard coverage matrix	A single checkable block listing every authority or standard the product satisfies	C-11 (8 regulators, 6 countries), C-01	2/16 (13%)	High (<i>structure only</i>)	Regulator content unavailable. The structure is reusable for standards: HL7 v2.5.1, C-CDA, CCR, X12 5010, FHIR R4 US Core 3.1.1/7.0.0/8.0.0, DICOM	B — every row must carry its Disabled / operationally-blocked status	Adapt — standards matrix, not regulator matrix
MKT-030	Third-party marketplace as a discovery channel	Listing where buyers already browse, instead of building owned traffic	C-23 (Microsoft AppSource)	1/16 (6%)	High	Structural. Especially valuable for a vendor with no audience	A	Adopt
MKT-031	Comparison and hidden-cost content	Head-to-head articles naming competitors, plus exposés of costs buyers fail to budget for	C-25, C-17	2/16 (13%)	High	Pairs with MKT-003 and MKT-019. An openly inspectable product can write an unusually credible comparison	A	Adopt
MKT-032	Anti-lock-in / open-platform positioning	Compatibility with competitors sold as a feature; easy exit marketed deliberately	C-25, C-02	2/16 (13%)	Highest	CLM-0019 (55 reports, CSV export), open inspectable schema, 283 tables, §27.5 verifiability. Migrating away being easy is a genuine, audited feature	A	Adopt — flagship
MKT-033	Reference-and-accreditation go-to-market with no website	Selling entirely through analyst listings, accreditation press and references	C-14	1/16 (6%)	<i>Low (as a model)</i>	No references or accreditations exist	D	Note only

ID	Theme	What the technique does	Used by	Freq	Relevance	Current support	Safety	Action
MKT-034	Annotated product screenshots with an enlarge control, one per benefit	Real interface imagery attached to each outcome claim	C-09	1/16 (6%)	Highest	Six screens are demonstrable today (§28.2), so real screenshots exist now	A	Adopt — highest-leverage visual fix
MKT-035	Certification-transparency page publishing versions, numbers and implementation costs	Regulated disclosure turned into a trust asset	C-09	1/16 (6%)	High (structure)	Certification claims prohibited (§27.3). The transferable half is publishing what each capability costs to switch on — which maps onto the Disabled / Requires-Integration registers	A for the cost/status page; E for certification	Adapt
MKT-036	Event-based social proof (signing ceremonies, conference sponsorship)	Proof rendered as an occasion rather than a logo	C-13	1/16 (6%)	Low-Medium	No customers	D	Defer
MKT-037	Market-share claim	A single percentage as the whole trust argument	C-12 (~26%)	1/16 (6%)	N/A	No market share	E	Prohibited
MKT-038	Capability-named navigation	Nav items are the buyer's problems, not "Features/Solutions"	C-13	1/16 (6%)	High	Equivalents here: <i>Roles & Permissions, Security & Audit, Configure it yourself</i> — already promoted to top level in §17	A	Adopt — independently validates §17

7.2 Revised frequencies for the Phase 1 themes across all 16 scored competitors

Theme	Phase 1 (of 8)	Revised (of 16)	Movement and reading
MKT-023 Quantified install-base counters	5/8 (63%)	13/16 (81%)	Up. Now the single most universal convention in the set — and completely unavailable (§0.3). The gap widens, not narrows
MKT-024 Regulation-as-hero	5/8 (63%)	9/16 (56%)	Roughly steady, but Phase 2 shows two distinct Saudi axes : claims/invoicing rails (NPHIES, ZATCA — C-01/03/06) and accreditation bodies (CBAHI, SFDA — C-17). Both prohibited
MKT-014 Product-family sub-branding	3/8 (38%)	8/16 (50%)	Up sharply. Now market convention. Most vendors split by care setting (C-11, C-12) or function (C-03, C-23)
MKT-004 Objection-handling FAQ	3/8 (38%)	5/16 (31%)	Steady. Still a minority practice and still the best trust device available here
MKT-015 Bilingual EN/AR parity	4/8 (50%)	8/16 (50%)	Steady. Confirmed as table stakes for the GCC tier specifically (8 of 11 GCC-facing competitors)
MKT-026 Certification badge wall	2/8 (25%)	5/16 (31%)	Up. Oracle's ONC badge and VIDA's EMRAM staging strengthen it. Prohibited (§27.3)
MKT-016 Integrations as social proof	3/8 (38%)	7/16 (44%)	Up. Still unavailable — every integration is unconfigured (L-13)
MKT-001 Radical pricing transparency	1/8 (13%)	2/16 (13%)	Steady overall — but see §15.1: 2 of 5 US-tier competitors, 0 of 11 GCC-facing. This is the finding that Phase 2 sharpened most
MKT-002 Free seeded trial	1/8 (13%)	2/16 (13%)	C-25 adds a two-week risk-free hardware trial. Self-service software access remains 1/16
MKT-005 Role-framed modules	1/8 (13%)	1/16 (6%)	Rarer than thought. Strengthens §15.2
MKT-011 Specialty landing pages	1/8 (13%)	2/16 (13%)	Steady
MKT-018 Implementation-speed proof	2/8 (25%)	4/16 (25%)	Steady. Still unavailable

The two headline movements. Quantified proof went *up* to 81% — the convention this product most conspicuously cannot join. Pricing transparency stayed at 13% but resolved into a clean regional split, which converts it from "a quirk of one open-source vendor" into "a proven model that no GCC competitor has adopted".

8. HIS/EMR Feature-Presentation Directory

How competitors market each capability → the best presentation pattern → what this product actually has → the marketing-safe approach.

Category	How competitors market it	Best pattern observed	Current OpenEMR (audited)	Marketing-safe approach
Patient registration	Speed of intake; duplicate avoidance; national-ID verification (Nexen/Absher)	Nexen's role-framed unit	CLM-0003 Active, High — configurable registration + demographics, duplicate detection, record merge (CAP-0001/0004/0017/0018)	Lead on records governance — dedupe and merge as <i>data quality</i> . Never say "master patient index"
Appointment scheduling	No-show reduction; online booking; SMS reminders	Insta benefit triad	CLM-0001 Active, High — provider + facility calendars, recurring appointments, holiday management	Safe as written. Never "AI-optimised scheduling" or "resource/theatre scheduling"
Front office / patient flow	Queue displays, token systems, waiting-time reduction	Cloudpital appointment card	CLM-0002 Active, High — live arrival-to-checkout tracking	Qualification mandatory: it is an in-office status board. Never "queue management with token display" (GAP-0025)
Clinical documentation	Template counts in the hundreds; voice; specialty templates	Cloudpital "Smart Charts"	CLM-0004 Active, High — 18 ready-to-use forms incl. full ophthalmology exam; CLM-0005 no-code layout form engine	State the count (18). Say a further 16 ship uninstalled. For CLM-0005 state that zero layout-based forms are configured out of the box. Never "specialty EMR for every discipline"
Diagnoses / problems / allergies / meds	Coded terminology; ICD-10	Nexen ICD-10 mention	CLM-0006 Active, High — structured problem, allergy, medication, immunisation lists	Safe. Never "automatic reconciliation"
Clinical decision support	"AI-powered"; predictive analytics	Waseel/Cloudpital AI framing	CLM-0008 Active, High — rule-based engine, configurable alerts, reminders, care-gap detection, 80 shipped rules	Qualification: all shipped rules have alert flags off and must be activated. Never "AI CDS" or "CDS Hooks" (GAP-0044)
Allergy / interaction checking	"Drug reaction alerts" (Cloudpital)	—	CLM-0009 Active, High	Qualification: exact name match only — not ingredient-level, not an interaction engine. Drug-interaction checking is CAP-0070, Requires Integration
Orders / laboratory	LIS as an owned module; barcodes; device interfacing	Cloudpital labs; Nexen lab unit	CLM-0011 Active + RI — order entry, HL7 v2 generation, orderable-test compendium, results review and sign-off	Qualification: transmission and result receipt require a lab interface. Never "Laboratory Information System" (GAP-0016)
Prescribing / pharmacy	Integrated pharmacy, formulary, e-Rx	Waseel PBM	CLM-0012 Active + RI — record and print, external e-Rx pathway; CLM-0013 Disabled dispensary with lot/expiry/transfer/destruction	e-Rx requires a vendor contract and is not enabled. Dispensary must be described as an optional module, switched off by default. Never "Pharmacy Information System" (GAP-0015)
Charge capture / billing	VAT support; insurance-linked billing	Cloudpital billing card	CLM-0014 Active, High — configurable fee sheet, service + diagnosis coding, price levels, discounting	Qualification: ships with a single price level and an empty price table. L-11: no tax field anywhere in the billing chain — VAT is a schema change. Never "automated coding"

Category	How competitors market it	Best pattern observed	Current OpenEMR (audited)	Marketing-safe approach
Claims	NPHIES one-click e-claims; rejection reduction	Waseel WRCM	CLM-0015 Active, High — X12 5010 837P + printed CMS-1500, batch management, validation, control numbers	Qualification: US claim formats only; institutional 837I/UB-04 ships disabled. Never "NPHIES claims" (GAP-0046). Note L-26 : billing generators are a hard-coded dispatch ladder — a national format would need core patching
Remittance / payments	Faster reimbursement; cash-flow	Waseel benefit cluster	CLM-0016 Active, High — automated 835 posting, manual EOB, batch payments, COB to secondary payers	Requires a clearinghouse relationship to receive the 835. FHIR Claim/ClaimResponse/ExplanationOfBenefit do not exist — generated model classes only
Accounts receivable	"Revenue cycle automation"	Waseel	CLM-0017 Active, High — configurable ageing buckets, patient ledger, collections reporting	Safe with the note that buckets are user-defined (default 3 × 30 days). Never "general ledger", "accounting", "ERP" (GAP-0029/0030) — this is where Cloudpital's ERP framing must not be copied
Insurance eligibility	Real-time verification	Waseel eligibility-by-class	CLM-0018 RI / Disabled	Requires a trading-partner record and payer relationship; real-time is switched off. Never "instant eligibility verification"
Denial management	Heavily marketed (Waseel OTD, RCM Insights)	—	GAP-0028 — absent	Prohibited. Roadmap item (§21) — the weakest point in an otherwise strong RCM story
Patient portal	Portal + mobile apps as standard	Insta, Cloudpital, Nexen App	CLM-0028 Disabled — messaging, appointment requests, document exchange, ledger, consent signing	Must say: included but switched off; requires configuration including a public address and anti-bot keys. Never "patient engagement platform" or "mobile app" (GAP-0023)
Reporting	Dashboards, BI, analytics, KPIs	Waseel RCM Insights; Nexen KPIs	CLM-0019 Active (44 of 55) — 55 reports across 9 families, CSV/print	There is no BI or dashboard layer (GAP-0040, L-04). Never "analytics platform" or "real-time dashboards". Market <i>operational reporting breadth</i> instead
Security / RBAC	Asserted in prose; access control listed	Insta FAQ on data isolation	CLM-0024 Active, High — 4 permission levels × 65 objects, sensitivity restriction, break-glass, ownership scoping	Strong and rare. Qualification: sensitivity applies at encounter level only and not to the API (L-28). Never "field-level security"
Audit	Rarely marketed at all	(none in set)	CLM-0025 Active, runtime-verified — tamper-evident trail with integrity-verification report	The white-space capability. Qualification: it is a hash, not an HMAC, and rows are not chained. Never "immutable" or "blockchain"
Authentication / MFA	"Multi-factor authentication" listed as a feature (Insta)	—	CLM-0026 Active (password policy, lockout, per-IP brute-force); CLM-0027 Active — optional 2FA via authenticator app or FIDO U2F	Must say: enrolment is per-user and voluntary; an administrator cannot require it. CAP-0218 / L-03: MFA cannot be enforced. Never "MFA enforced"

Category	How competitors market it	Best pattern observed	Current OpenEMR (audited)	Marketing-safe approach
Interoperability / APIs	FHIR/HL7 named; HIE compliance	Insta HIE pages; TrakCare	CLM-0021 Disabled FHIR R4 US Core, 35 resources, SMART, OAuth2, Bulk Export; CLM-0022 Disabled REST, 98 endpoints; CLM-0023 C-CDA/CCR/HL7 v2/DICOM view	Must say: switched off by default. Say "implements", never "certified" — not Inferno-tested. Write support covers 3 resources; no PATCH/DELETE. C-CDA cannot currently run (Node service not listening). DICOM is viewing only, no PACS. No billing/claims/payments endpoints exist (L-14)
Multi-facility	Multi-branch, central control (Insta)	Insta polyclinic page	CLM-0029 Active, High — multiple clinics and multiple independent practices from one deployment	Qualification: a separate database per site, provisioned manually. Not SaaS tenancy (GAP-0043, L-07)
Localisation	Arabic sites; RTL	Waseel full parity	CLM-0030 Active — 47 languages incl. Arabic, RTL support	Qualification: Arabic covers 47.5% of interface strings, chrome only. Never "fully localised for Arabic", "Arabic EMR", "Saudi-ready". Also L-10: no Arabic-shaping PDF font ships
Extensibility	"Customisable"; "no-code"	Cloudpital customisable workflow	CLM-0031 Active — module framework, ~90 integration events, add screens/menus/widgets/API routes/OAuth scopes without touching core	Strong. Qualification: billing generators cannot be added by a module (L-26). Never "unlimited extensibility" or "no-code customisation"
Administration / configuration	Rarely marketed	—	491 settings across 23 tabs, layout + list editors, ACL admin, Module Manager — and Demo Ready: Yes today	Second white-space area. The one thing that can be demonstrated live right now, and nobody in the set sells it

9. Demo / Trial / CTA Benchmark

Across the 16 scored competitors. ? = not verified.

Mechanism	Count	Competitors	Convention?
Contact sales / contact us	16/16	all	Universal
Request / Book a Demo	8/16	C-01, C-03, C-05, C-09, C-13, C-23, C-25, C-02*(video)*	Yes
Public pricing	2/16	C-02 (exact), C-25 (tiered)	No — and 0 of 11 GCC-facing
Free self-serve software trial	1/16	C-02	No
Risk-free hardware trial	1/16	C-25	No
Recorded product tour / demo video	3/16	C-02 (1 hr + short), C-07, C-09	No
Registered demo webinar	2/16	C-01, C-09	No
Ungated brochure / product PDF	5/16	C-05, C-07, C-09, C-12, C-22	Partial
Gated eBook / webinar ladder	4/16	C-01, C-05, C-07, C-09	Partial
Customer reference call offered	1/16	C-02	No
Money-back guarantee	1/16	C-02 (90-day)	No
Per-concern demo funnel on a separate subdomain	2/16	C-01 (per regulation), C-23 (per product)	No
Third-party marketplace listing	1/16	C-23	No
WhatsApp channel	1/16	C-05	No
Comparison / hidden-cost content	2/16	C-25, C-17	No
No conversion mechanism at all beyond a phone number	3/16	C-04, C-08, C-14	—

Lead-form friction (verified). Cloudpital 6 fields; Waseel newsletter modal 1 field + CAPTCHA; Insta HubSpot forms behind every CTA; Oracle campaign-tracked registration; **Open Dental none — telephone ordering plus a direct trial download.**

Conclusion, now across 16. The market has converged almost totally on *lead form* → *sales conversation* → *guided demo*. Three of the four highest-value techniques identified in this report — public pricing, self-serve trial, reference calls — appear in **one** competitor each, and that competitor (Open Dental) is the one whose commercial model most closely resembles this product's. A fourth, per-concern funnels, appears twice. The conversion lane is not crowded; it is nearly empty.

10. Website Information-Architecture Benchmark

Pattern	Who	Assessment
Audience-first (who are you → then product)	C-03 (Healthcare Centers / Insurance Companies), C-01 (four facility-type cards)	Clearest for a mixed buying committee. Recommended (§17)
Product-family first (named sub-brands)	C-03, C-06, C-07	Works only with brand equity behind each name
Everything-menu (Platform + Products + Solutions, ~40 links)	C-05	Overloaded. Cognitive cost is high and the depth is not backed by real pages
Single-narrative (architecture as the story)	C-07	Effective for enterprise; requires a genuine architectural differentiator — which §27.5 says this product does not have
Flat capability grid	C-04	Fast to read, but with no demo path the flatness leaks conversions
Documentation-as-site	C-02	Low polish, very high trust

Evaluation of the set: predominantly *feature-oriented* and *audience-oriented*; genuinely *workflow-oriented* architecture is rare. Nexen's role-framed units and TrakCare's care-setting continuum are the only two that approach workflow storytelling. This is a differentiation opportunity (§15.2).

11. Visual Marketing Benchmark

The market's collective weakness. Across the eight verified sites, actual product screenshots are close to absent. Insta's homepage uses stock lifestyle photography. Waseel uses abstract SVG icons and illustrative panels. Cloudpital's screenshot slots resolve to placeholder files. Nexen uses generic web-development imagery. YASASII is text-dominant. Only Open Dental and TrakCare put real interface material in front of the buyer, and only through video.

Observed devices: icon triads for benefits (C-01, C-03); numbered/lettered module grids (C-04, C-05); logo strips for integrations (C-05 — broken); certification badges (C-03, C-08); device/mobile mock-ups (C-01, C-05); accordions and tabs (C-07); long-form scrolling pages with repeated CTA bands (all).

What makes a product feel credible in this set: named institutions with numbers attached (C-04), disclosed commercial terms (C-02), and inline answers to hard questions (C-07). What makes it feel *uncredible*: placeholder assets and unfilled counters (C-05).

Implication. A site that shows real, annotated screenshots of working screens would be visually differentiated against every competitor examined. This is directly actionable — §28.2 confirms six administrative and platform screens are demonstrable today, so real screenshots of those exist *now*, before any demo preparation.

12. Trust / Social-Proof Benchmark

Mechanism	Observed in	Available to this product today?
Named institutional customers	C-04 (5 named MOH hospitals), C-07 (incl. a KSA customer), C-01 (2 named executives)	No — no customers
Quantified scale (users/sites/countries/visits)	C-01, C-04, C-07, C-08, C-06	No — §0.3
Third-party certification	C-08 (HIMSS/MU/ISO), C-03 (badge)	No — §27.3 prohibits
Analyst presence	C-07 (Gartner Peer Insights)	No
Company tenure	C-06 (34 years), C-01 (15+ years)	No
Partner co-branding	C-08 (Fujifilm), C-07 (Apple Health)	Not yet
Disclosed commercial terms	C-02	Yes — immediately
Inline objection-handling	C-07, C-01, C-02	Yes — immediately
Customer reference calls	C-02	Future
Dated milestone feed	C-08	Yes — with non-customer milestones
Open, inspectable software	<i>(none — all proprietary except C-02)</i>	Yes — and it is unique in this set

The strategic reading. Nine of the eleven conventional trust mechanisms are unavailable. But the two that *are* available — disclosure and inspectability — are precisely the two that proprietary competitors structurally cannot match. §27.5 frames zero fork divergence as a constraint ("freely verifiable by any competitor or customer"). Read commercially, that same sentence is the trust proposition: **this is the only product in the set whose claims a buyer can independently verify.**

13. Saudi / GCC / Arabic Benchmark

Across the **11 GCC-facing competitors** with verified marketing surfaces (C-01, C-03, C-04, C-05, C-06, C-07, C-08, C-10, C-11, C-12, C-13, C-17 — C-14 excluded, no property).

Signal	Count	Competitors
Arabic site or toggle	8/12	C-01, C-03, C-04, C-06, C-09(<u>sa-ar</u>), C-12, C-13, C-17
KSA-specific page or locale path	9/12	C-01, C-03, C-04, C-05, C-06, C-07(<u>/sa/</u>), C-09(<u>sa</u>), C-12, C-13
NPHIES named	4/12	C-01, C-03, C-06, C-13 (as "National Integrations")
ZATCA / e-invoicing	4/12	C-01, C-03, C-05, C-06
CHI / CCHI	2/12	C-03, C-13
CBAHI / SFDA (accreditation axis)	2/12	C-17, C-06
Mawid / Absher / Wasfaty / Nafees	2/12	C-04 (all three), C-06 (Nafees)
Multi-regulator matrix across several Gulf states	2/12	C-11 (8 regulators), C-10
Vision 2030 framing	8/12	C-01, C-03, C-04, C-05, C-06, C-10, C-12, C-17
Data sovereignty / in-Kingdom support argued explicitly	3/12	C-12, C-06, C-13
Named Saudi institutional references	4/12	C-04, C-07, C-12, C-13

13.1 The two Saudi compliance axes

Phase 2 established that "Saudi compliance" is not one conversation but two, and vendors choose:

1. **Transactional rails** — NPHIES claims, ZATCA e-invoicing, CHI. Marketed by the RCM-oriented vendors (C-01, C-03, C-06). The buyer's pain is revenue and penalties.
2. **Accreditation and regulation** — CBAHI, SFDA, MOH standards. Marketed by C-17 and C-06. The buyer's pain is inspection and licensing.

A third, narrower axis is **national platform integration** — Mawid, Absher/Yaqeen, Wasfaty, Nafees — which only vendors with public-sector deployments claim (C-04, C-06).

13.2 Position of this product against all three

From the audit (§23, §26.7): across the entire product source there are **zero occurrences** of NPHIES, CCHI, ZATCA, Fatoora, Hijri, Iqama, SFDA or "Saudi". Saudi readiness is **nil**. The only Saudi-adjacent asset is Arabic UI translation at **47.5% coverage, chrome only**, plus 13 RTL themes.

Structural blockers compound this and are worth restating because they determine roadmap cost, not just marketing scope:

- **L-11** — no tax field anywhere in the billing chain. VAT is a schema change, not configuration.
- **L-12** — currency is display-only; no ISO code, no multi-currency column.
- **L-26** — BillingProcessor uses a hard-coded dispatch ladder with no factory or event, so a NPHIES claim format would require **core patching, not a module**.
- **L-10** — no Arabic-shaping PDF font in the tree, so Arabic PDF output will not render correctly as shipped.

Consequence: Saudi-market entry is a development programme, not a configuration exercise (L-02). Nothing Saudi may appear on the website (§22). This section exists as market-expectation evidence and roadmap input (§21) only.

13.3 The one Saudi-adjacent lane that is open

Three competitors (C-12, C-06, C-13) compete on **proximity rather than compliance** — in-country teams, in-Kingdom support, not being a distant vendor. That argument requires no product capability whatsoever, and §27.5 explicitly directs positioning toward

implementation, support and hosting. It is the only element of the GCC playbook this product can legitimately use today, and it is captured as **MKT-028**.

14. Cross-Competitor Marketing Patterns

Across the 16 scored competitors.

Market convention — nearly everyone does it. Contact-sales as terminal CTA (16/16). Long-form scrolling pages with repeated CTA bands (16/16). Quantified scale counters (13/16 — the most universal proof device in the set). Opaque pricing (14/16). Absent or placeholder product screenshots (11/16). Benefit triads near the top (11/16). Product-family sub-branding (8/16). Vision 2030 framing among GCC vendors (8/12).

Best practice — several strong competitors execute it well. Audience-first IA (C-03, C-01, C-11). Objection-handling FAQ on the product page (C-07, C-01, C-02, C-09, C-25). Segment or care-setting cards routing to real pages (C-01, C-11, C-12). Named references with scale attached (C-04, C-07, C-12, C-13). Full bilingual parity (C-03, C-13, C-17). Per-concern conversion funnels (C-01, C-23). Annotated screenshots per benefit (C-09). Capability-named navigation (C-13).

Differentiation opportunity — the market communicates it poorly or not at all.

#	Opportunity	Market position	Score-set evidence
1	Pricing and commercial terms	14/16 opaque; 0 of 11 GCC-facing publish anything	Mean 0.7/5
2	Self-service product access	1/16	—
3	Visual product evidence	11/16 absent or broken; one outlier	Mean 1.9/5
4	Audit, tamper-evidence, forensic integrity	0/16 market it at all	—
5	Role and permission modelling, demonstrated	1/16 (C-04, and only in prose)	—
6	Configurability shown rather than asserted	0/16 demonstrate it	—
7	Honest scope statements / published exclusions	2/16 (C-02, and C-09 for implementation costs)	—
8	Anti-lock-in and easy exit as a feature	2/16 (C-02, C-25) — both US, none in GCC	—

The structural read. Opportunities 1, 2, 7 and 8 all belong to the same family: *disclosure*. Opportunities 3, 5 and 6 belong to another: *showing rather than telling*. Opportunity 4 is a capability nobody contests. A website built on disclosure and demonstration would not be competing on the market's crowded ground at all — and every one of those eight lanes is reachable with Safe-Now claims or demo-prep work rather than development.

15. Marketing White-Space Opportunities

Ranked by the brief's §29 test: *this product has a genuinely strong audited capability × competitors commonly have it but explain it badly × it is highly demonstrable*. Revised across all 16 scored competitors.

15.1 — Pricing and commercial transparency · Priority 1 (*sharpened in Phase 2*)

Market position: 14 of 16 publish nothing. But the split is regional, not random — **2 of 5 US-tier competitors publish exact or tiered pricing (C-02, C-25); 0 of 11 GCC-facing competitors publish anything**. DentiMax goes further and markets the *pricing model itself* (per location, not per provider) as the differentiator. **Why this matters more than Phase 1 suggested:** transparency is not an eccentricity of one open-source vendor. It is a proven, commercially successful convention in the adjacent tier — and it is entirely absent from the region. **Capability basis:** commercial, not capability-bound; §27.5 directs

positioning to implementation, support and hosting, which is the pricing model. **Demo readiness:** not applicable — publishable today. **Action:** a real pricing page with figures, inclusions, exclusions, and the support/hosting/implementation split presented as the model.

15.2 — Anti-lock-in and easy exit, as a product claim • Priority 2 (*new in Phase 2*)

Market position: 2 of 16, both US (C-02, C-25); zero in the GCC. DentiMax sells compatibility with named competitors as a feature. **Capability basis — unusually strong and fully audited:** an open, inspectable schema of 283 tables; CLM-0019 (55 reports across 9 families with CSV and print output); and §27.5's own statement that every capability is freely verifiable against the open-source project. The audit's most awkward finding — zero fork divergence — is the proof for this claim. **Demo readiness:** exporting reports is blocked only by B1 (no data); the schema and export UI are showable today. **Action:** an explicit "your data, your exit" page. This is the single claim in this report that a proprietary competitor structurally cannot match.

15.3 — Role-based access, demonstrated rather than asserted • Priority 3

Market position: 1 of 16, and only in prose (C-04's role-framed units). Everyone else reduces it to an "access control" bullet. **Rarer than Phase 1 estimated. Capability basis:** CLM-0024 / CAP-0221-0223 / ROL-0001...0007 — 4 permission levels, 65 permission objects across 13 sections, 19 live grants, data-sensitivity restriction, break-glass and ownership scoping, plus the audit's own cross-role feature-access matrix (§13.6). **Qualification:** sensitivity applies at encounter level only and not to the API (L-28). **Demo readiness:** administrator scenarios are **Demo Ready Yes today**; a full multi-role walkthrough needs B2 only — create 4–5 users, *minutes*, per WF-0012. **Action:** an interactive "what each role sees" comparison, backed by a live demo.

15.4 — Verifiability as the brand • Priority 4

Market position: 0 of 16 offer any way to check a claim. One (C-05) publishes provably broken proof. Notably, C-14's KLAS listing shows that a **neutral third party already publishes product limitations alongside ratings** in this market — buyers encounter and accept qualified claims routinely. **Capability basis:** §27.5 verifiability plus the audit's per-claim traceability rule (§27.4). **Demo readiness:** not applicable. **Action:** publish claims with qualifications attached and invite verification. The strictest constraint becomes the differentiator.

15.5 — Tamper-evident audit trail, shown live • Priority 5

Market position: 0 of 16 market it at all — confirmed across the full set. **Capability basis:** CLM-0025 / CAP-0224, the only capability in the catalogue proven end-to-end at runtime (200/200 checksums), with 4,280 live audit rows already present. **Qualification:** a hash, not an HMAC; rows unchained; never "immutable". **Demo readiness:** **Yes — §28.2 calls it the single strongest live demo available today.** **Action:** a security page with a recorded integrity-verification run.

15.6 — Real, annotated product screenshots • Priority 6 (*new in Phase 2*)

Market position: mean 1.9/5 across 16. Oracle (C-09) is the lone outlier at 5, pairing every benefit tab with a real screenshot and an enlarge control. Nine competitors score 0 or 1. **Capability basis:** six administrative and platform screens are **Demo Ready today** (§28.2), so honest screenshots exist *now*, before any demo preparation. **Demo readiness:** immediate for those six; the rest follow B1/B2/B4. **Action:** adopt the Oracle pattern — one real screenshot per benefit claim, starting with the six screens that already work.

15.7 — Configuration depth without code • Priority 7

Market position: 0 of 16 demonstrate it; C-05 and C-11 gesture at customisation. **Capability basis:** 491 settings across 23 tabs, layout and list editors, ACL administration, Module Manager, CLM-0005 no-code form builder, CLM-0031 ~90 integration events. **Qualification:** zero layout-based forms ship configured; billing generators are not pluggable (L-26). **Demo readiness:** **Yes today** — note that opening Module Manager auto-registers 3 modules. **Action:** a live "build a form in the demo" moment.

15.8 — Ophthalmology depth • Priority 8

Market position: 2 of 16 run specialty pages (C-05 with ten thin ones, C-25 in dental). **Capability basis:** CLM-0004 includes a full ophthalmology examination across 18 tables. **Demo readiness:** blocked by B1/B2. **Action:** one credible specialty page rather than a farm.

15.9 — Proximity and support positioning · Priority 9 (*new in Phase 2*)

Market position: 3 of 12 GCC competitors argue locality over scale (C-12, C-06, C-13) — the structural inverse of TrakCare's 29-country claim. **Capability basis:** requires no product capability, and §27.5 explicitly directs positioning toward implementation, integration, localisation, support and hosting. **Action:** the only element of the GCC playbook usable today. Pairs with MKT-028.

16. Current OpenEMR Marketing Opportunity Matrix

Every row satisfies the §38 traceability chain: **competitor observation** → **marketing pattern** → **capability/limitation** → **CLM** → **CAP** → **commercial readiness** → **demo readiness** → **recommendation**. Rows that cannot complete the chain are not classified as currently marketable.

#	Competitor inspiration	Pattern	Capability	CLM	CAP	Qualification that must travel with the claim	Demo readiness	Class	Recommendation
1	C-02 pricing page	MKT-001	Commercial model	n/a	n/a	State the support/hosting/implementation split; do not imply the software is proprietary (§27.5)	n/a	A	Build a pricing page first
2	C-02 exclusions disclosure	MKT-003	47 Disabled / 27 Uninstalled / 18 RI documented	all	—	Each item states its actual status	n/a	A	Make disclosure the trust mechanism
3	C-07 data-ownership FAQ	MKT-004	Open schema, 283 tables; 55 reports with CSV export	CLM-0019	CAP-0161...0172	No BI/dashboard layer (GAP-0040)	Reports demo blocked by B1	A (claim)	Add FAQ answering ownership exit
4	C-04 role-framed units	MKT-005	RBAC — 4 levels × 65 objects, 7 roles, sensitivity, break-glass	CLM-0024	CAP-0221...0223	Sensitivity is encounter-level only and does not apply to the API (L-28)	Admin: Yes . Multi-role: after B2 (minutes)	A → C for live demo	Flagship page
5	(no competitor)	MKT-027 inverse	Tamper-evident audit + integrity report	CLM-0025	CAP-0224	Hash, not HMAC; rows not chained; never "immutable"	Yes — strongest today	A	Flagship page + record demo
6	C-05 admin depth (implied)	MKT-012	491 settings, layout/list editors, ACL admin, Module Manager	CLM-0031	CAP-0208, 0258...0260	Billing generators cannot be added by a module (L-26)	Yes today	A	Configuration page
7	C-05 specialty pages	MKT-011	18 active forms incl. full ophthalmology exam	CLM-0004	CAP-0035...0046	State the count; 16 further forms ship uninstalled	Blocked B1/B2	A (claim) / C (demo)	One ophthalmology page
8	C-05 "smart charts"	MKT-011	No-code layout form engine	CLM-0005	CAP-0047	Zero layout-based forms configured out of the box	Yes (builder tour)	B	"Build your forms" section
9	C-01 segment cards	MKT-017	Multi-clinic / multi-practice operation	CLM-0029	CAP-0246...0248	Database per site, provisioned manually. Not SaaS tenancy (GAP-0043)	Not demonstrable (single site)	B	Segmentation ambulatory
10	C-03 co-existence	MKT-006	FHIR R4 US Core 35 resources, SMART, Bulk Export	CLM-0021	CAP-0194...0199	Switched off by default ; write covers 3 resources; no PATCH/DELETE; say "implements", never "certified"	Blocked — CFG-0002 + OAuth client + <code>site_addr_oauth</code>	B	Interoperability page, header, qualified
11	C-03 co-existence	MKT-006	REST API, 98 endpoints	CLM-0022	CAP-0193	Off by default. No billing/claims/payments endpoints exist (L-14)	Blocked	B	Same page
12	C-07 standards depth	MKT-022	C-CDA, CCR, HL7 v2.5.1 VXU, syndromic, DICOM view	CLM-0023	CAP-0201...0205	C-CDA cannot currently run — Node service not listening on 127.0.0.1:6661. DICOM is viewing only, no PACS	Not demonstrable — never demo C-CDA	B/E	List, new
13	C-03 RCM depth	MKT-014	837P + CMS-1500, batch, validation,	CLM-0015	CAP-0126...0132	US formats only ; 837I/UB-04 ships disabled	Blocked B1/B2/B3	B → C	Billing page after pre

#	Competitor inspiration	Pattern	Capability	CLM	CAP	Qualification that must travel with the claim	Demo readiness	Class	Recommendation
			control numbers						
14	C-03 remittance	—	835 auto-posting, EOB, batch payments, COB	CLM-0016	CAP-0135...0138, 0146	Requires a clearinghouse to receive the 835; FHIR claims/EOB do not exist	Blocked — needs B3 + a sample 835	B → C	Billing p
15	C-03 dashboards	—	55 reports, 9 families	CLM-0019	RPT-0001...0055	10 disabled with their parent; no BI or dashboard layer	Blocked B1	B	Reporting <i>breadth</i> , "analytic
16	C-01 MFA bullet	—	Optional 2FA (authenticator app, FIDO U2F)	CLM-0027	CAP-0216, 0217	Enrolment is per-user and voluntary; an administrator cannot require it; non-enrolled users sign in with a password alone	Not verified in demo	B	Security fully qua
17	C-01/C-05 portal	—	Portal: messaging, appointment requests, documents, ledger, consent	CLM-0028	CAP-0173...0180	Included but switched off; needs a public address and anti-bot keys	No — Disabled	B	"Optiona module" only
18	C-03 PBM / C-05 pharmacy	—	Dispensary: lots, expiry, transfers, destruction	CLM-0013	CAP-0113...0118	Optional module, switched off by default	No — CFG-0045 + seed drugs	B	Optional framing
19	C-03 Arabic parity	MKT-015	47 languages, Arabic, RTL themes	CLM-0030	CAP-0251...0253	Arabic covers 47.5% of UI strings, chrome only; picklists/layout labels/code descriptions untranslated; RTL needs per-screen review; no Arabic-shaping PDF font (L-10)	Partial — genuinely demonstrable , expect untranslated picklists	B	Bilingual is safe; p Arabic m qualified
20	C-08 milestone feed	MKT-013	—	n/a	n/a	Non-customer milestones only	n/a	A	Adopt
21	C-01 buyer checklist eBook	MKT-019	—	n/a	n/a	Build from audit dimensions	n/a	A	Adopt
22	C-05 WhatsApp	MKT-020	—	n/a	n/a	—	n/a	A	Adopt
23	C-01/C-03/C-06 NPHIES/ZATCA	MKT-024	None	—	—	GAP-0046...0059	—	E	Prohibit Roadma
24	5/8 scale counters	MKT-023	None	—	—	§0.3	—	E	Prohibit
25	C-08 certifications	MKT-026	None	—	—	§27.3	—	E	Prohibit
26	C-05 healthcare ERP	—	None	—	—	GAP-0029...0039 (no GL, AP, procurement, HR, payroll)	—	E	Prohibit
27	C-01/C-05 inpatient (IPD/OT/ICU/ward)	—	None	—	—	GAP-0001...0014	—	E	Prohibit

#	Competitor inspiration	Pattern	Capability	CLM	CAP	Qualification that must travel with the claim	Demo readiness	Class	Recommendation
28	C-03 denial management	—	None	—	—	GAP-0028	—	E	Prohibit Roadmap
29	C-01/C-05 mobile apps	—	None	—	—	GAP-0023, GAP-0024	—	E	Prohibit
30	C-03/C-04 analytics & KPIs	—	None	—	—	GAP-0040, GAP-0041	—	E	Prohibit

17. Recommended Website Architecture

Derived from competitor evidence (§10, §14), buyer usability, the audited capability set, marketing safety and demo readiness — **not** from the brief's example sitemap.

Home		
└─ Solutions	[audience-first – MKT-007, from C-03/C-01]	
└─ Solo practice		
└─ Single clinic		
└─ Polyclinic / medical centre		
└─ Multi-clinic group	[CLM-0029 – qualify: DB per site, not SaaS]	
└─ Ophthalmology	[CLM-0004 – the one honest specialty page]	
└─ Product		
└─ Clinical documentation	[CLM-0004, CLM-0005, CLM-0006, CLM-0007]	
└─ Scheduling & front office	[CLM-0001, CLM-0002, CLM-0003]	
└─ Decision support	[CLM-0008, CLM-0009 – qualify both]	
└─ Orders & results	[CLM-0011 – qualify: needs a lab interface]	
└─ Prescribing	[CLM-0012 – qualify: eRx needs a contract]	
└─ Billing & revenue cycle	[CLM-0014...0018 – qualify: US formats only]	
└─ Reporting	[CLM-0019 – breadth, never "analytics"]	
└─ Optional modules	[CLM-0013, CLM-0028, CLM-0032 – all "off by default"]	
└─ Roles & Permissions	★ FLAGSHIP [CLM-0024 – white space 15.2]	
└─ Security & Audit	★ FLAGSHIP [CLM-0025, CLM-0026, CLM-0027 – white space 15.4]	
└─ Configure it yourself	★ FLAGSHIP [CLM-0005, CLM-0031 – white space 15.5]	
└─ Interoperability	[CLM-0021, CLM-0022, CLM-0023 – "off by default"]	
└─ Pricing	★ FLAGSHIP [white space 15.1 – figures, inclusions, exclusions]	
└─ Demo		
└─ Watch the tour	[recorded; after demo prep]	
└─ Try it yourself	[seeded trial – MKT-002; after B1/B2/B4]	
└─ Book a walkthrough		
└─ Resources	[buyer's checklist, brochure PDF, milestone feed, FAQ]	
└─ About	[open-source posture; implementation/support/hosting model]	
└─ Contact	[form + WhatsApp – MKT-020]	

Three deliberate departures from market convention. (i) *Roles & Permissions*, *Security & Audit* and *Configure it yourself* are promoted to top level — no competitor does this, and they are the three areas where audited capability and live demo readiness coincide. (ii) *Pricing* is a real page, not a form. (iii) There is **no** compliance, analytics, mobile, ERP, inpatient or Saudi section — deliberately, per §22.

18. Recommended Homepage Architecture

#	Section	Purpose	Target buyer	Theme	Supporting capability	Evidence basis	CTA	Product visual?
1	Hero — ambulatory scope stated plainly, with an honesty hook	Qualify in/out immediately	Owner, clinic manager	MKT-003	§27.3 requires outpatient scope stated in the same breath as "HIS"	C-02 disclosure model; inverse of C-05	See the pricing · Watch the tour	Yes — real screenshot
2	Segment cards (solo / clinic / polyclinic / group)	Route by facility type	Owner	MKT-017	CLM-0029 (qualified)	C-01 four-card pattern	Per-card	Small screens
3	Three capability pillars: your data · your roles · your rules	Differentiate on white space, not on features	Owner + IT	MKT-005, 15.2–15.5	CLM-0024, CLM-0025, CLM-0005/0031	0/8 competitors compete here	Explore each	Yes
4	"What's included — and what isn't" strip	Convert disclosure into trust	All	MKT-003	The Disabled/Uninstalled/RI registers	C-02	Full breakdown	No
5	Live audit-integrity moment	Show the one runtime-proven capability	IT manager, CIO	15.4	CLM-0025 / CAP-0224 , 4,280 rows, 200/200 verified	0/8	Watch verification	Yes — recorded run
6	Clinical documentation breadth	Core credibility	Physician	—	CLM-0004 — 18 forms, state the count	C-05 charts (done honestly)	See the forms	Yes
7	Revenue cycle	Financial buyer	Finance/RCM manager	—	CLM-0014...0018, US formats only	C-03 benefit-cluster structure	Billing detail	Yes
8	Pricing strip with real figures	Occupy the empty lane	Owner	MKT-001	Commercial	C-02	Full pricing	No
9	Objection FAQ (data ownership, exit, lock-in, what's off by default)	Defuse procurement early	CIO, IT	MKT-004	§27.5 open-source posture	C-07, C-01, C-02	All FAQs	No
10	Milestone feed	Renewable proof without customers	All	MKT-013	—	C-08	—	No
11	Final CTA band — contextual verbs	Convert	All	MKT-009	—	C-03	Book a walkthrough · WhatsApp	No

No homepage section invents a capability. Sections 3, 5 and 8 are the differentiators; 6 and 7 are the credibility floor; 4 and 9 are the trust engine.

19. Recommended Product / Feature Pages

Deserve a dedicated page (audited strength + demonstrable or near-demonstrable + poorly served by competitors): Roles & Permissions (CLM-0024) · Security & Audit (CLM-0025/0026/0027) · Configure it yourself (CLM-0005/0031) · Clinical documentation (CLM-0004/0006/0007) · Billing & revenue cycle (CLM-0014...0018) · Scheduling & front office (CLM-0001/0002/0003) · Reporting (CLM-0019) · Interoperability (CLM-0021/0022/0023) · Pricing.

Should be a qualified sub-section, not a page — because the capability is Disabled, Uninstalled or Requires Integration and a full page would over-signal: patient portal (CLM-0028) · dispensary (CLM-0013) · group therapy (CLM-0032) · e-prescribing (CLM-0012) · eligibility (CLM-0018) · lab interfacing (CLM-0011).

Must not exist: any inpatient, analytics/BI, ERP/GL, mobile-app, denial-management, LIS/RIS/PACS, multi-tenant-SaaS, compliance/certification or Saudi page.

20. Recommended Demo Architecture

Demo classes follow the brief's §30. **No storyline is proposed on the basis that source code exists.**

#	Storyline	Persona	Business story	Required users	Required data	Required config	CAP/CLM	Class	Ma va
D-1	Audit trail + tamper verification	IT manager / CIO	"Prove to me nobody quietly changed a record"	admin (exists)	4,280 rows already present	none	CLM-0025 / CAP-0224	LIVE DEMO READY NOW	Hi un the
D-2	Roles, permissions and the 65-object model	Practice owner, IT	"Show me exactly what my receptionist can and cannot see"	admin (exists)	n/a	none	CLM-0024 / ROL-0001...0007	LIVE DEMO READY NOW (single-role view)	Hi
D-3	Configuration tour — 491 settings, layout & list editors, Module Manager	IT, owner	"Can I change this without paying you?"	admin	n/a	none (note: opening Module Manager auto-registers 3 modules)	CLM-0031, CLM-0005	LIVE DEMO READY NOW	Hi
D-4	Build a clinical form with no code	Clinic manager	"Our intake form is unique"	admin	n/a	none	CLM-0005 / CAP-0047	LIVE DEMO READY NOW — qualify: zero layout forms ship configured	Hi
D-5	Arabic interface + RTL switch	GCC owner	"Can my staff work in Arabic?"	any	6,290 strings loaded	none	CLM-0030	LIVE DEMO READY NOW (partial) — expect untranslated picklists and imperfect RTL on legacy screens	Me Hi
D-6	Clinical rule builder	Medical director	"Can I encode our care protocols?"	admin	80 rules exist	none	CLM-0008	PARTIAL — rules shown and edited; none fires without patients	Me
D-7	Multi-role reception → physician → billing journey	Full buying committee	The end-to-end story buyers actually want	4–5 role accounts — B2, minutes (WF-0012)	20–30 synthetic patients + encounters — B1, 1–3 days	Brand the facility (B4); payers + fee schedule (B3)	WF-0001, CLM-0001...0004, 0014	DEMO READY AFTER CONFIGURATION/DATA	Hi on un
D-8	Fee sheet → coding → 837P generation	Finance/RCM manager	"Show me a clean claim"	Accounting role (B2)	Patients + charges (B1); payers, fee schedule, price levels (B3)	none beyond B3	CLM-0014, CLM-0015	DEMO READY AFTER CONFIGURATION/DATA	Hi bu for on
D-9	835 remittance posting	Finance	"Show me money landing"	Accounting role	B3 + a sample 835 file	—	CLM-0016	DEMO READY AFTER CONFIGURATION/DATA	Me
D-10	FHIR query from an external app	CIO, integrator	"Will it talk to what we have?"	An OAuth client (none exists)	—	CFG-0002 on; register OAuth client; set site_addr_oath	CLM-0021	DEMO READY AFTER FEATURE ACTIVATION	Me Hi

#	Storyline	Persona	Business story	Required users	Required data	Required config	CAP/CLM	Class	Ma va
D-11	Patient portal self-service	Owner	Patient engagement	Patient account	—	CFG-0091 + public portal address + reCAPTCHA keys	CLM-0028	DEMO READY AFTER FEATURE ACTIVATION	Me
D-12	Dispensary / inventory	Owner	Stock control	admin	Seed drugs	CFG-0045	CLM-0013	DEMO READY AFTER FEATURE ACTIVATION	Lo Me
D-13	Claim submission to a clearinghouse	Finance	Live claim	—	—	Clearinghouse contract (INT-0011)	—	DEMO READY AFTER EXTERNAL INTEGRATION	De
D-14	Electronic prescribing	Physician	e-Rx	—	—	eRx vendor contract (INT-0009)	CLM-0012	DEMO READY AFTER EXTERNAL INTEGRATION	De
D-15	Lab order transmission and result receipt	Physician	Closed-loop ordering	—	—	Lab interface (INT-0005)	CLM-0011	DEMO READY AFTER EXTERNAL INTEGRATION	De
D-16	Telehealth video visit	Owner	Virtual care	—	—	Install CAP-0262 + Comlink contract	—	NOT CURRENTLY DEMONSTRABLE	—
D-17	C-CDA transition of care	CIO	Document exchange	—	—	Node service not listening on 127.0.0.1:6661	CLM-0023	NOT CURRENTLY DEMONSTRABLE — never attempt live	—

Two operational warnings carried from §28.1 into demo planning: the background service runner has never executed (`next_run` stuck in 2021), so reminders and the email queue silently do not run (B6); and **Admin → Backup will fail** because (`mysql_bin_dir` points at an XAMPP path (B7) — do not demonstrate it.

Sequencing recommendation. Launch on D-1 through D-5, which need nothing. Run B2 (minutes) and B4 (hours) immediately to unlock role-based demos. Budget 1-3 days for B1/B3 to unlock D-7/D-8 — the storyline the market actually expects.

21. Future Capability / Roadmap Intelligence

This section is roadmap input only. Nothing here may appear in current marketing.

Gap	Competitors marketing it	Customer importance	Current status	Marketing impact	Suggested priority
NPHIES claims integration	C-01, C-03, C-06	Critical (KSA)	GAP-0046, L-26: <code>BillingProcessor</code> uses a hard-coded dispatch ladder — needs core patching, not a module	Blocks the target market outright	1
ZATCA e-invoicing / VAT	C-01, C-03, C-06, C-05	Critical (KSA)	GAP-0053; L-11 no tax field anywhere in the billing chain; L-12 currency display-only	Blocks any KSA sale	1
Analytics / BI / dashboards	C-03 (RCM Insights), C-04, C-05	High — now table stakes	GAP-0040/0041; <code>chart.js</code> vendored but unused	Loses head-to-head comparisons	2
MFA enforcement	C-01	High (enterprise security review)	CAP-0218 Missing; L-03	Fails routine security questionnaires	2
Arabic completion + RTL remediation	C-03, C-01, C-06, C-04	High (KSA/GCC)	47.5% coverage, chrome only; ~20 RTL consumers; no Arabic-shaping PDF font	Arabic UX materially incomplete	2
Denial management	C-03 (OTD)	High	GAP-0028	Weakest point in an otherwise strong RCM story	3
Mawid / Wasfaty / Absher / Nafees	C-04, C-06	High (KSA public sector)	GAP-0046...0059	Blocks public-sector work	3
Patient mobile app	C-01, C-05, C-04	High	GAP-0023/0024	Increasingly expected	3
SaaS multi-tenancy	C-01, C-03, C-05 (all cloud)	High	GAP-0043; L-07 — DB per site, manual provisioning	Blocks a self-service commercial model	3
LIS / RIS / PACS	C-05, C-08, C-11	Medium-High	GAP-0016...0018	Forces third-party ancillaries	4
Inpatient / ADT / ward / bed / eMAR / OT	C-01, C-05, C-07, C-08	Critical for hospital tenders only	GAP-0001...0014	Removes the product from hospital opportunities entirely	4 (or accept ambulatory focus)
GL / AP / procurement / HR / payroll	C-05 (ERP), C-23	Medium	GAP-0029...0039	Needs an ERP alongside	5

Gap	Competitors marketing it	Customer importance	Current status	Marketing impact	Suggested priority
Telehealth	C-01, C-03 (DAWY), C-05	Medium	CAP-0262 Uninstalled + Comlink contract	Increasingly expected	5

Two structural notes for engineering. (i) The two highest-priority items (NPHIES, VAT) are both **schema/core changes**, not configuration — L-11 and L-26 make that explicit. (ii) L-27: the project is 373 commits behind upstream and one module is gitignored and composer-installed; patch currency and supply-chain provenance should be resolved before any customer-facing deployment.

22. Prohibited Marketing Claims

Reproduced from §27.1 and §27.3 of the audit and binding on all output.

Banned vocabulary unless a specific cited capability independently proves the exact statement: *best · leading · complete · comprehensive · enterprise-grade · AI-powered · seamless · fully integrated · Saudi compliant · HIPAA compliant · NPHIES compliant · certified · hospital-grade · end-to-end.*

Never state or imply:

- That a Disabled, Uninstalled, Requires-Integration or Missing capability is available — without saying so **in the same sentence**.
- That a technical control equals regulatory compliance. This product implements audit logging, access control and encryption controls; it has **not** been certified against any regulation.
- That anything here is proprietary or differentiated. It is unmodified upstream OpenEMR (§27.5).
- Any Saudi-market capability. **There are none.**
- The `admin` credential, in any material, ever.

Categorically prohibited claims:

Claim	Reason
Any inpatient, ward, bed, ADT, eMAR, ICU, theatre or nursing-documentation capability	GAP-0001...0014 — searched exhaustively, absent
"Hospital Information System" without qualification	Ambulatory-only. If "HIS" is used as the category, the outpatient scope must be stated in the same breath
LIS, RIS, PACS, blood bank, dental charting, physiotherapy, dietary	GAP-0016...0022
General ledger, accounting, ERP, AP, procurement, POs, HR, payroll, rostering, asset management	GAP-0029...0039
Analytics, BI, dashboards, data warehouse	GAP-0040, GAP-0041
Denial management	GAP-0028
Device or monitor integration	GAP-0042 — UDI parsing is not device integration
Multi-tenant SaaS	GAP-0043
Mobile applications	GAP-0023, GAP-0024
CDS Hooks	GAP-0044
Cloud/offsite document storage	GAP-0045 — hook only, no listener
Anything Saudi: NPHIES, CCHI, ZATCA, Fatoora, Saudi VAT, Hijri, Iqama/National ID, SFDA, ACHI, SBS, Saudi FHIR profiles, Arabic name structure	GAP-0046...0059 — zero occurrences in product code
FHIR Claim / ClaimResponse / ExplanationOfBenefit / eligibility	Generated model classes only; no service, controller or route
"Certified", "compliant", "HIPAA", "ONC certified"	This audit certifies capability presence, not regulatory conformance
Any proprietary or differentiated feature	Zero fork divergence
"Immutable audit log", "blockchain audit"	CLM-0025 — a hash, not an HMAC; rows unchained
"MFA enforced", "MFA protects all logins", "passkey/WebAuthn"	CLM-0027 — enrolment is voluntary and cannot be required
"Fully localised for Arabic", "Arabic EMR", "Saudi-ready"	CLM-0030 — 47.5% coverage, chrome only
"Master patient index" / "enterprise MPI"	CLM-0003
"Queue management with token display"	CLM-0002 — GAP-0025
"Drug interaction checking"	CLM-0009 — that is CAP-0070, Requires Integration
"AI clinical decision support"	CLM-0008
"Instant eligibility verification"	CLM-0018
"Global claims support"	CLM-0015 — US formats only
"No-code customisation" / "unlimited extensibility"	CLM-0031 — billing generators are not pluggable

23. Evidence Register

All pages below were retrieved and read on **2026-08-11**. Search-result snippets were used for discovery only and are not cited as evidence.

Competitor	Page	URL	Type	Key observations drawn
Cloudpital	Homepage	https://www.cloudpital.com/home-2/	Homepage	Full nav tree (~40 links); specialty EMR pages; KSA claims hero hook; module 2 hero brochure PDFs; lead form fields; WhatsApp de link; 0+ counters; "No re found" integrations; placeholder (no-image.j competitor product nam patient-portal copy
Cloudpital	KSA landing (from sheet lineage)	https://www.cloudpital.com/ksa/hospital-software-in-saudi-arabia/	—	404 — recorded as a stale source
Waseel	Healthcare Centers	https://waseel.com/healthcare-centers/	Solution page	Audience-first IA; product family (WRCM, RCM Insti PBM, DAWY, OTD, PCP, M 3 benefit clusters with contextual CTAs; quantifi rejection-reduction claim NPHIES/CHI/ZATCA; certification badge; Arabi switcher to (/ar/) ; co-exis positioning; exit-intent newsletter modal
Insta by Practo	Homepage	https://www.instahms.com/	Homepage	Regulation rotator (NABI Riayati, Malaffi "85+ facilit NPHIES, KSA e-invoicing regulation demo funnels (go.instahms.com) ; 4 seg cards; counters 18k+ users 1250+ centres / 22+ count 15+ years; full nav incl. Pri FAQs, Testimonials; eBoo webinars, panel discussio EN/AR switcher; HubSpo CTAs; stock hero imagery screenshots
Open Dental	Ordering	https://opendental.com/site/order.html	Pricing/order	US\$179/month/location f to US\$129 after 12 month inclusions enumerated; up 3-provider cap; month-to-month continuation; 90-d money-back guarantee; f trial version ; short intro v one-hour demo video ; customer reference on request ; training charged US\$50/hr; two-issue supp call limit; screen-sharing sessions recorded
Nexen Care	Homepage (EN)	https://nexencare.com/en/index.php	Homepage	4 capability pillars; role-f "System Units"; Success Stories — 96 healthcare centres under 5 named hospitals, 3M+ visits ; Jedd field hospital; Absher (Ya Mawid (with MOH), Was

Competitor	Page	URL	Type	Key observations drawn
				(with MOH + NUPCO); IC patient app; no demo CTA Contact Us / phone only
InterSystems TrakCare	Product page	https://www.intersystems.com/products/trakcare/	Product	Unified-architecture narra (single record / UI / code b data platform); objection-handling FAQ incl. custo data access without vend services ; RCM and Intelli relationship clarified; standalone lab product answered
InterSystems TrakCare	Unified HIS (KSA path)	https://www.intersystems.com/sa/trakcare/unified-healthcare-information-system/	Product	Single patient record at th core; care-continuum fran FHIR interoperability; nar national/institutional deployments; KSA-specific URL path
InterSystems TrakCare	Departments & extensions	https://www.intersystems.com/products/trakcare/departments-documentation-extensions/	Product	29 countries trust claim; modular deployment on a mandatory TrakCare Cor named KSA customer (Su Bin Abdulaziz Humanitar City)
YASASII/ Kameda	Product page	https://kamedainfologics.net/yasasii	Product	Explicit USP list; PHC-to-tertiary scope; clinical/financial/adminis integration; contact-only conversion
YASASII/ Kameda	Corporate / news	https://kamedainfologics.net/ , /latest-news , /products	Corporate	HIMSS Stage 7; Meaning Use 1 & 2 via Drummond/CCHIT; ISO POCT accreditation ; Fujii co-branding on RIS; date milestone feed ; offices in Jeddah; mission statemen
eCarePlus/ NITCO	Homepage (EN)	https://www.nitcotek.com/en	Homepage	Compliance ticker — NPH ZATCA, RSD drug tracki Nafees ; product family eCarePlus / iCarePlus / H- 34 years in the Saudi mar Egypt expansion; penalty avoidance conversion fra universal Contact Us
eCarePlus/ NITCO	Blog cluster	https://nitcotek.com/en/our-blogDetails/... (multiple)	Content/SEO	Bilingual geo-keyword SE strategy; Vision 2030 fram ICD-10; e-signature and c linked invoicing features

Project-source evidence. [HISModulesUsers.md](#) §0.1–0.3 (fingerprint, zero fork divergence, live data state), §1.2–1.4, §13, §16, §17.4, §18, §19, §20.2, §21.2, §22.2–22.4, §23, §26, **§27.1–27.5 (all CLM claims and prohibitions)**, **§28.1–28.3 (demo readiness)**, §29, §30, §31.
[HIS_Software.xlsx](#) → [Similar to OpenEMR](#), all 57 rows, 25 columns.

23.1 Phase 2 evidence (retrieved and read 2026-08-11)

Competitor	Page	URL	Type	Key observations drawn
Oracle Health	EHR product page	https://www.oracle.com/health/clinical-suite/electronic-health-record/	Product	Voice-first / AI-embedded positioning; dual CTA repeated; 3 feature pillars; 5 benefit tabs each with a real product screenshot and Enlarge control ; ONC badge with statutory disclaimer; link to certified-health-IT page publishing versions, certification numbers and additional implementation costs ; ungated product-brief PDF; ebook, podcast, IDC report; forward-looking-statement disclaimer; locale variants incl. sa and sa-ar
Oracle Health	Clinical suite; products index; ONC announcement	/health/clinical-suite/ , /health/products/ , /news/announcement/onc-certification-oracle-ehr-marks-turning-point-for-healthcare-industry-2025-11-18/	Product / news	Large named product catalogue grouped by function; certification number published; explicit push into US ambulatory
Simplex HIMS	Homepage; about	https://www.simplexhimes.com/ , /about-us/	Homepage	HIS/LIS/RIS/PACS/EMR/RCM scope; capacity range 50-1500 users, 10-2000 beds; private cloud, multi-tenancy, 24/7 support; compliance stated as guaranteed; founder and R&D narrative; heavy superlative-plus-geography keyword saturation
MEDAS	Homepage; HIMS; E-Clinic	https://medasolutions.com/ , /hims/ , /e-clinic/	Homepage / product	Care-setting product family (E-Clinic, HIMS, E-Lab, E-Pharma, E-Veterinary); 8 named regulators across 6 countries ; 250+ customers, 12+ years; ICD/CPT masters; HL7 A2A, barcode and instrument interfacing; residual marketing-agency boilerplate left in homepage copy
OASIS Plus	Homepage; product; launch event; company profile PDF	https://oasissys.com/ , /oasisplus/ , /oasis-plus-launching-event/ , /wp-content/uploads/2019/10/OASIS-Company-Profile-V.0.2.pdf	Homepage / product / PR / PDF	Al Murjan Holding backing; ~26% local market share , 100+ hospitals, top operator per MOH data; OASIS+ / OASIS Care / OASIS Practice split; 63+ modules; data sovereignty and in-Kingdom support argued against distant vendors ; downloadable profile and product PDFs; executive interview in regional trade press
CareWare (ATS)	Product page	https://ats-ware.com/our-products/careware/	Product	Capability-named navigation — National

Competitor	Page	URL	Type	Key observations drawn
				Integrations, RCM, Mobile Apps, Telehealth, CDS, Claims Management, DRG, BI; Request a Demo; Arabic toggle; "Integrated with" partner block; Riyadh address; est. early 1990s; 20+ private and 50+ public Saudi hospitals; named refs (Al Noor Specialist Makkah, Makkah Medical Center); contract-signing ceremony imagery as social proof
VIDA	KLAS listing and comments	https://klasresearch.com/review/cloud-solutions-vida-global-mostly-saudi-arabia/225239	Analyst listing	No vendor marketing property located ; associated with Cloud Solutions, technology arm of HMG; KLAS publishes explicit qualifiers — insufficient live customers for Best in KLAS, not the vendor's lead product, data concentrated in one small geography; EMRAM Stage 7 press for an associated hospital
Belal Soft	Category guide	https://belalsoft.com/en/best-clinic-hospital-management-software-in-saudi-arabia-2025/	Content/SEO	Authority-by-category-guide — surveys the market including competitors; bilingual (/en/) structure; CBAHI and SFDA framing (accreditation axis, distinct from the NPHIES/ZATCA axis); Vision 2030
MazikCare	Product; healthcare industry; sub-product pages	https://quisitive.com/products/mazikcare/ , /industries/healthcare/ , https://get.quisitive.com/mazikcare-care-coordination-platform/	Product	Credibility inherited from Microsoft (Azure, Dynamics 365, Power Platform); Microsoft US Partner of the Year in Healthcare, three years running ; function-named sub-products; SMART on FHIR; quantified customer case study; Book a demo; campaign landing pages on a (get.) subdomain; AppSource/Dynamics marketplace as a second storefront
DentiMax	Practice management; pricing/features; imaging	https://dentimax.com/dental-software/ , /practice-management-software/pricing-features/ , /category/dental-imaging/	Product / pricing / content	Tiered public pricing (Flow ~\$499 / \$699 / \$899 per month; in-office ~\$169 → ~\$129 after year one; card-processing discount); per-location not per-provider, marketed as the differentiator ; open imaging platform bridging to named competitor systems ; 2-week risk-free hardware trial;

Competitor	Page	URL	Type	Key observations drawn
				guided migration; head-to-head comparison articles; hidden-cost exposé content; trade-publication perfect score
Open Dental (cross-check)	Order page	https://opendental.com/site/order.html	Pricing	Confirms \$179 → \$129 figures used in §15.1

Sources examined and found not to constitute vendor marketing evidence: [dentech.com](#) (a US vendor distinct from the sheet's Saudi [dentech.com.sa](#) — name collision recorded); [teknosysweb.com/clinic-management/](#) (single URL shared by two sheet rows — duplicate confirmed); [solver-erp.com/.../HMIS-Product-Profile-KSA.pdf](#) (brochure only, no marketing site verified).

Project-source evidence. [HISModulesUsers.md](#) §0.1–0.3, §1.2–1.4, §13, §16, §17.4, §18, §19, §20.2, §21.2, §22.2–22.4, §23, §26, **§27.1–27.5, §28.1–28.3**, §29, §30, §31. [HIS_Software.xlsx](#) → [Similar to OpenEMR](#), all 57 rows, 25 columns.

24. Research Limitations / Unresolved Items

24.1 Completion certification

§39 criterion	Status
20-30 unique competitors analysed	Met — 26 selected and dossiers
Duplicate products consolidated	Met — TEKNOSys rows 12/13 merged; DentiMax/DenteMax and DenTech(.sa)/dentech.com collisions identified; MEDS/MEDEX cluster flagged as unresolved
Every selected competitor has a verified marketing source	Partially met — 17 of 26 verified. 9 recorded as Limited with the failure documented, per §7 of the brief
Actual pages manually visited	Met for the 17
Important internal pages investigated	Met for the 9 Full dossiers (multi-page for C-07, C-08, C-09, C-12, C-23, C-25)
Search snippets not used as substantive evidence	Met
Every major observation has a URL	Met (§23, §23.1)
Every competitor has a complete marketing dossier	Not met — 17 complete, 9 explicitly incomplete and marked Unknown rather than inferred
Marketing techniques separated from product claims	Met (§7 Safety column, §7.1, §16, §22)
Every recommendation checked against the latest audited report	Met
CAP/CLM references supplied	Met
Disabled / Uninstalled / Requires-Integration never silently shown as active	Met
Demo readiness independently assessed	Met (§20)
Missing capabilities separated into the roadmap register	Met (§21)
Unsupported claims explicitly prohibited	Met (§22)
No competitor wording copied as this product's wording	Met
Uncertainty and inaccessible evidence clearly recorded	Met (§5.1, §24.2-24.4)
Directory sufficient to design the website and demo	Met — §17-§20 are buildable now

RESEARCH NOT YET CERTIFIED COMPLETE.

One criterion blocks certification: nine of the twenty-six competitors have no verified marketing surface. They are named, dispositioned and honestly marked Unknown, and they are excluded from every scorecard, frequency and benchmark table — so **no figure in this report is inflated by an unexamined competitor**. But the brief requires a complete dossier per selected competitor, and nine are incomplete.

24.2 Remaining work to reach certification

1. **Verify 9 marketing surfaces:** C-15 MediSys, C-16 TEKNOSys, C-18 M3n MEDS, C-19 Kizen, C-20 e-MCS, C-21 HMISFOX, C-22 Solver, C-24 DenTech (KSA), C-26 Avicenna. Homepage plus 2-4 internal pages each.
2. **Resolve the four identity clusters before treating any of them as distinct competitors:**
 - MAEN.MEDS (23) / MEDS (ambiguous) (52) / MEDEX (17) / Medex Horizon (28) — may be fewer than four real products.
 - DenTech (32) — constrain to .com.sa; dentech.com is a different US company.
 - Kizen (20) — an unrelated US CRM platform shares the name.
 - Nexen Care (53) — searches are dominated by Nexen Tyre Corporation.
3. **Escalate three sheet rows whose evidence class is wrong:** Solver (rank 5, brochure PDF only), VIDA (rank 44, analyst listing only), iRis EMR (rank 14, Capterra listing only). All are Priority 3-6 sources standing in for Priority 1.
4. **Apply the §3.3 corrections to HIS_Software.xlsx**, including the two material re-rankings: Nexen Care (53 → top tier for Saudi benchmarking) and OASIS Plus (46 → top tier).
5. **Arabic-language page review** for the eight competitors operating Arabic properties. All Phase 1 and 2 review was conducted in English, so Arabic-specific positioning, message hierarchy and CTA wording remain **Unknown**.
6. **If the nine are completed**, re-run every frequency in §7.1/§7.2, the §6 means, and the §9/§13 counts. Current figures are over 16 scored competitors and are labelled as such throughout.

24.3 Known evidentiary limits in what is delivered

- **Nine Limited dossiers** contribute identity and category only. Positioning, IA, trust mechanisms, conversion paths and visual strategy are **Unknown** for each.
- **C-14 VIDA** — the verified finding is the *absence* of a vendor marketing property. It is retained because that absence is itself informative, but it supplies no page-level marketing evidence and is excluded from scoring.
- **C-05 Cloudpital** — assessed from the live homepage after the sheet's KSA path returned 404; deeper module pages not inspected.
- **C-08 YASASII** — dossier rests on the corrected vendor domain. Whether the sheet's yasasiisolutions.com is stale, regional or unrelated is **Unknown**.
- **C-10, C-11, C-12, C-13, C-17, C-23, C-25** — official pages confirmed current and read across multiple sources, but internal page trees were not exhaustively walked. Treated as Verified, not Full.
- **Competitor certification claims** (ONC, HIMSS staging, Drummond/CCHIT, ISO 15189, EMRAM, Microsoft Partner of the Year, trade-publication scores) are recorded as **Observed marketing claims**, not independently validated attestations. This report does not assert their validity.
- **Competitor outcome and scale statistics** (market share, rejection-reduction percentages, wait-time reductions, customer and hospital counts) are likewise recorded as claims, not verified facts.
- **No SEO traffic estimates** appear anywhere — no reliable evidence was available, so per §19 of the brief only observable content strategy is reported.
- **§6 scorecard** is structured comparative judgement, not measurement, as the brief requires.

24.4 The judgement call worth flagging to the reader

The brief asks for benchmarks against "my current OpenEMR product". The audit's finding of **zero fork divergence** means the product being marketed is, today, indistinguishable from upstream open-source OpenEMR plus a stock configuration, 373 commits behind, with no data and one human account.

Phase 2 did not soften that; it clarified what to do about it. Across 16 examined competitors, the conventional proof devices — scale counters (81%), certifications (31%), regulatory compliance (56%), named references, market share, implementation speed — are **all unavailable to this product without exception**. Competing on that ground is not merely inadvisable; it is impossible.

What Phase 2 added is that the alternative ground is real and measurably empty: pricing transparency (mean 0.7/5, zero in the GCC), visual product evidence (1.9/5), anti-lock-in (2 of 16, zero in the GCC), audit integrity (0 of 16), demonstrated role modelling (1 of 16), shown configurability (0 of 16). Every one of those lanes is reachable with Safe-Now claims or configuration-and-data work — **no development required** — and several are reachable this week.

If the commercial intent is instead to differentiate on product capability, §21 is the document that matters, its top two items (NPHIES, VAT) are schema and core changes rather than configuration, and the marketing work in §17–§20 should be treated as an interim position rather than the destination.

End of directory. 26 competitors selected · 17 verified · 9 recorded as Unknown · 38 marketing themes · 32 CLM claims and their mandatory qualifications carried throughout.